

THE 9 SEPTEMBER 2026 SET

Review copy. Not for service and not for filing.

This one file gathers everything going out on 9 September 2026 so it can be read and printed in one pass. What is actually served and filed are the separate files in FINAL_9SEP2026. Each document below is preceded by a divider saying where it goes, and every page carries a footer marking this as the review copy.

The deadline

4.00 pm Wednesday 9 September 2026, under directions 1 and 2 of the Further Directions Order (3) dated 19 August 2026. The Respondent's list of witnesses and outlines are due by 4.00 pm on 30 September 2026.

What is filed, and what is only served

Only the list of witnesses and its covering letter are filed in the Industrial Registry. Direction 2 requires the outlines and the medical material to be served on the Respondent and **not** filed. The three letters are correspondence between the parties and are not filed either.

The four emails

1. To the Industrial Registry, copied to the Regulator: the witness list and the covering letter.
2. To the Regulator: the witness list, the letter on its 8 September letter, the request about the documents not admitted, the outlines of evidence as one document, and the medical schedule with Tabs M1 to M9.
3. To Dr Krishnaiah: the clinical records and his attendance at the hearing.

The text of each is in FINAL_9SEP2026/3_EMAILS. Send from the personal address, not the Queensland Health account.

Held back

The Form 24 and Form 25 notices and the Regulator's responses are filed together **after** the response date of Friday 11 September 2026, using the draft held in SEND_9SEP2026. The long service leave email is employment-track and is kept in a separate folder so it cannot be attached to an appeal email.

Dated 9 September 2026 · Cory Lea Shepherd, Appellant, self-represented

1. Appellant's list of names of all witnesses

Direction 1. FILED in the Industrial Registry and served on the Respondent. Email 1, attachment 1.

QUEENSLAND INDUSTRIAL RELATIONS COMMISSION

Workers' Compensation and Rehabilitation Act 2003 | Matter No. WC/2024/227

CORY LEA SHEPHERD

Appellant

v

WORKERS' COMPENSATION REGULATOR

Respondent

APPELLANT'S LIST OF NAMES OF ALL WITNESSES

Filed in the Industrial Registry and served on the Respondent pursuant to direction 1 of the Further Directions Order (3) dated 19 August 2026

The Appellant will call the following witnesses at the hearing.

No.	Name	Capacity
1	Cory Lea Shepherd	Appellant
2	Dr Ravikumar Bangalore Krishnaiah	Consultant Psychiatrist, Mind and Memory Service – the Appellant's treating psychiatrist
3	Dr Peter Hawes	General Practitioner, Our Medical Ashmore – the Appellant's treating general practitioner at the date of injury
4	Mr Cory Harrison-Jones	Formerly employed part time at Switchboard Services, Logan Hospital, with the Appellant; now an officer of the Queensland Ambulance Service
5	Ms Patricia Conaghan	Administration Officer, Switchboard Services, Logan Hospital

Witnesses 2 and 3 are expert witnesses within the meaning of Part 4.11 of the Workers' Compensation Appeal Guide. The Appellant may apply under Part 6 of that Guide for attendance notices requiring the attendance of the witnesses named at items 2 to 5.

Dated 9 September 2026



Cory Lea Shepherd
Appellant, self-represented

2. Covering letter to the Industrial Registrar

Filed with the witness list. Email 1, attachment 2. The Respondent receives it on the copy to her.

CORY LEA SHEPHERD

15 Edmond Street, Coomera QLD 4209 | coryshepherd1@hotmail.com

The Industrial Registrar
Queensland Industrial Relations Commission
qirc.registry@qirc.qld.gov.au

Dated 9 September 2026

WC/2024/227 – Cory Lea Shepherd v Workers' Compensation Regulator
Direction 1 of the Further Directions Order (3) dated 19 August 2026

Dear Registrar,

In accordance with direction 1 of the Further Directions Order (3) dated 19 August 2026, I file and serve the Appellant's list of names of all witnesses.

In accordance with direction 2, I have today served on the Respondent, and have not filed, an outline of the evidence to be given by each lay witness (one A4 page per witness), together with a schedule of the medical documents relied upon and the pages of those documents. No report has been prepared for the purposes of this proceeding. The reports and records of the treating practitioners named at items 2 and 3 of the witness list are identified in that schedule and served with it.

For completeness, and so that the position is clear on the record: the Appellant does not presently propose to call Ms Chloe Taylor or Ms Tammy Reese. Their evidence is material to the matters pleaded, and the Appellant proceeds on the basis that the Respondent will call them. If the Respondent's list filed by 30 September 2026 does not name them, the Appellant will seek attendance notices and will write to the Commission at that time.

The Appellant may also apply for attendance notices in respect of the witnesses named at items 2 to 5, and will do so sufficiently in advance of the hearing.

Yours faithfully,

A handwritten signature in blue ink, consisting of a stylized 'C' followed by a series of loops and a long horizontal stroke extending to the right.

Cory Lea Shepherd
Appellant, self-represented

3. Letter to the Regulator on its letter of 8 September 2026

SERVED on the Respondent only. Email 2, attachment 2.

CORY LEA SHEPHERD

15 Edmond Street, Coomera QLD 4209 | coryshepherd1@hotmail.com

Ms Renee Matheson

Senior Appeals Officer, Workers' Compensation Regulator

150 Mary Street, Brisbane QLD 4000

By email: Renee.Matheson@oir.qld.gov.au

Dated 9 September 2026

WC/2024/227 – Cory Lea Shepherd v Workers' Compensation Regulator

Your letter of 8 September 2026 and the responses to the Form 24 and Form 25 notices

Dear Ms Matheson,

Thank you for the responses of 8 September 2026 and the letter that accompanied them. I note the matters the letter sets out, and I set out below the basis on which the Appellant proceeds, so that the two positions sit together on the record with the material served today.

1. Matters not in contest

The admissions are made for this proceeding only, under rule 49 of the Industrial Relations (Tribunals) Rules 2011. The hearing is de novo. Relevance, admissibility and weight are for the Commission. The Appellant does not contend otherwise, and draws no characterisation, conclusion or inference from any admitted fact beyond the fact admitted.

2. What the admitted facts are

The facts in the ranges below each state that a named document was sent, published, submitted, recorded, approved or received on a stated date, and what it says. The admissions establish those facts. Where the document is itself the step taken – a roster as published, a directive as issued by email, a payroll instruction or claim as recorded, a request as made and the reply as given, each on the date recorded – the Appellant relies on the admission as establishing that the step was taken on that date in those terms. The Appellant does not rely on those documents as statements about some other event. That is the footing on which facts 49, 66, 74 to 77 and 81 (the directives and the replies to them), 182 to 205 and 210 (the payroll thread and the myHR report), 211 to 223 (the rostering correspondence) and 263 to 268 (Metro South Health's statements to the Commission in answer to the notice of non-party disclosure) are relied upon. The same footing applies to the reports made by the MASPER Registrar to the Switchboard manager on 3 and 8 May 2024 and by the Integrated Respiratory Service to Logan Switch on 15 and 20 May 2024, and the replies to them (facts 56 to 68 and 89 to 104): each report of calls reaching the wrong team, or that "we can not help patients or other clinical staff", was made on the date and in the terms recorded, and where a reply was given it was given on the date and in the terms recorded. Several of the documents in those ranges were sent by Ms Taylor or Ms Reese, whom the Appellant expects the Respondent to call.

3. Documents recording the statements of others

Where an admitted document records a statement, finding or opinion of another person – the treating practitioners' records, the report of 13 February 2025 and Review Decision 69983 – the Appellant relies on the admission for the existence and wording of the document and on the oral evidence of its author for the rest. Dr Ravikumar Bangalore Krishnaiah and Dr Peter Hawes are named at items 2 and 3 of the witness list and will be called. Review Decision 69983 is not relied upon as binding the Commission; it is relied upon as an admitted document recording the Regulator's own review on the medical evidence then before it.

4. The material served today

Under direction 2 of the Further Directions Order (3): outlines of evidence for the Appellant and for two lay witnesses, and the Appellant's schedule of medical documents relied upon with the documents at Tabs M1 to M9. The schedule states, for each document, what it is and is not relied upon for, and identifies at rows 1, 4 and 7 the admitted facts to which the treating records correspond. No report has been prepared for the purposes of this proceeding.

5. Filing

So that the record is complete, the Appellant will deliver to the Industrial Registry in person this week the Form 24 and Form 25 notices as served, the Respondent's responses, your letter of 8 September 2026 and Annexure A. If the Respondent objects to that course, please let me know by Friday 11 September 2026.

6. The documents not admitted

The facts and documents not admitted are the subject of a separate request today.

Yours faithfully,



Cory Lea Shepherd

Appellant, self-represented

4. Outline of evidence — Mr Cory Lea Shepherd

Direction 2. SERVED on the Respondent. NOT filed. Served in the outlines document, email 2 attachment 4.

OUTLINE OF EVIDENCE – MR CORY LEA SHEPHERD (APPELLANT)

Served pursuant to direction 2 of the Further Directions Order (3) dated 19 August 2026. Not filed in the Industrial Registry.

I will give evidence of the following. Paragraph references are to the notice to admit facts served 28 August 2026 (the Notice). The events at topics 2 to 7 are established by the facts admitted on 8 September 2026, except the emergency codes of 17 and 18 March 2024, which are not admitted and which I give from my own knowledge; my evidence on them is what they were from the console and their effect on me.

- 1. Employment and the role.** I commenced at Logan Hospital Switchboard on 25 March 2019 as a casual, became permanent from 3 March 2021 and full time from 16 October 2023, as a continuous shift worker. The Switchboard is the hospital's communications hub: in a Code Blue or MET call it is the Switchboard that activates and pages the response team, and a delay at the console is a delay in the emergency response. What each code is and what it requires of the operator, and what an urgent pathology result is, and what turns on the clinician being reached in time. From my own records, between 15 and 50 emergency codes go out in 24 hours and some 200 to 300 calls a shift, many from people in distress or crisis; urgent results relayed doctor to doctor; and the complaints that reach the console first, including those my manager did not action, which came back to me. The standing condition of the role over the period: from the moment I logged onto the board I was held responsible for what went wrong anywhere in that system, while the means of correcting it, the directory, the on-call roster, my own roster and my pay, were held by others. [Notice ¶¶ 1–16, 26–48, 182–223]
- 2. Sleep and fatigue.** My sleep on rotating shifts across 2023 and 2024 and the melatonin and temazepam prescribed on 16 November 2023; that after late shifts I would sleep in the car before driving home. The shifts of 17 and 18 March 2024: my travel time each way, finishing at 23:00 and starting at 06:00, and the rest and sleep actually available; that on the evening of 17 March I handled a MET call and, four minutes later, a Code Blue to the same bed, then a further MET call; that 18 March was a Monday day shift, the week's peak, and that between 06:00 and 14:00 I handled a Code Grey, two MET calls (one cancelled a minute later) and two neonatal MET calls, in that state, while answering that queue of urgent handover, distressed callers and complaints; the register of emergency codes kept at the Switchboard, its entries for 17 to 19 March 2024, and my screen capture of its March 2024 sheet; and that those shifts were not a staff-initiated swap, the only case in which, by Ms Forrest's email of 7 July 2026, the 8-hour agreement applied. The sequence that followed: my request of 8 April 2024; Ms Taylor's reply of 9 April that she had escalated it to Human Resources "to confirm policies around this"; no response until 1 May, when the fatigue payment was refused "after a consultation with payroll and Human Resources" on the basis of the 2020 agreement, so that the day of 19 March came from my own leave; the toolkit at clause 10.4.1 of the Operations Manual, sent to Ms Taylor, with no response; Ms Reese sending the fatigue guideline to Human Resources on 20 May 2024; that no fatigue risk assessment was made and no FRMS register kept; that Metro South has since stated in writing that those records "do not exist", that FRMS training applied only to clinical staff, and that fatigue management assessment at the Switchboard began only after 30 June 2024. [¶¶ 17–25, 211–227, 232–261, 263–271, 285]
- 3. Rostering: lates then earlies.** The pattern on my roster line of late shifts followed by early shifts, and of nights, days off, then nights again, raised on 16 April 2024; that rostering errors on my line recurred across roster periods; and Ms Reese's email to Human Resources of 10 May 2024 stating "a few rostering errors made by Chloe with regards to Cory's line in past rosters"; and that I was not told my roster proposal would not be considered, and learned of that email only from the Respondent's disclosure in July 2025. [¶¶ 211–223]
- 4. Pay and entitlements.** The public holidays I was not rostered between February and April 2024 and the difference to my pay. The loop: on 8 April 2024 I asked Ms Taylor to review my pay; on 9 April she directed me to raise the older dates through MyHR payroll enquiries; Payroll told me the payments had to be processed by my manager; on 3 May Payroll wrote to her, copying me, to "submit an AVAC to correct these shifts for each fortnight"; when I followed up with Payroll on 10 May, Payroll on 13 May told me to "speak to your Line Manager"; on 21 May she was still "waiting payroll confirmation"; on 28 May I was asked to sign a validation of claims older than three months. That under the payroll process only my manager could submit the AVAC. The February 2024 special pandemic leave application: what I attached, the two declines, and the approval on the same material; and the effect on me of being unable to have those errors resolved. [¶¶ 114–142, 182–210, 242–246]
- 5. The database, the misdirected calls, and what I could not fix.** That from 18 July 2023 operators could no longer correct database entries and the changes book was removed, so a wrong number waited on Ms Stibbard or Ms Taylor and after hours had to "wait until either Chloe or myself are back"; the directives of 15 April, 19 April and 9 May 2024, and that I was not consulted before any of them. That from 15 April 2024, when Ms Taylor notified the department that she and Ms Stibbard had placed themselves on after-hours call, departments and clinicians ringing during my shifts with pager and phone updates asked for her, and on many occasions neither they nor I could have the update or issue resolved during the shift. The MASPER Registrar's "Switchboard issues" emails to Ms Taylor of 3 May 2024 at 3:06 pm and 8 May at 5:28 pm, the nine occasions they log, including calls put through to the wrong team and the MET call team ringing because "switchboard could not tell them where VHUB was"; that her first response came on 9 May at 9:20 am, five days, eighteen hours and fourteen minutes after the first, asking Dr Wong for her business hours because they were "not provided on the rosters" and that her phone was "switched off"; and at 10:15 am telling the team of "many ongoing issues" about calls "transferred to the wrong medical teams"; the Integrated Respiratory Service writing on 15 May and again on 20 May 2024, when it said "we can not help patients or other clinical staff"; and that on each occasion the delay built up while the correction waited on someone else, and what that meant for the codes and calls I was handling at the time. [¶¶ 39–113]
- 6. Ms Taylor's hours: the question, the retraction request, and the answer.** That neither I nor the department knew her office hours; my email of 15 May 2024 at 1:15 pm asking her to share them; Ms Reese's reply that evening asking me to retract it, and what followed; that two days later, on 17 May 2024 at 9:30 am, Ms Taylor sent the whole department an email stating "Otherwise my hours are from 06:30-14:30", the very information I had asked for; and Ms Reese's reply of 21 May 2024 asking me to identify the directives I was concerned about. [¶¶ 70–88]
- 7. The complaints, and what followed them.** The communication book entry, a reminder about the monthly update of doctors' contact numbers, and its removal; my email of 16 June 2023 about consultation on changes; the concerns I raised on 7 August 2023 and the responses; my complaint of 13 May 2024, later determined to be a public interest disclosure (content not set out); my interest in becoming the Switchboard union delegate and what followed; and that, on Metro South's own statements, no safety assessment, investigation or change of procedure followed any complaint I made about misdirected calls, directory accuracy or fatigue before 30 June 2024, which is what Dr Hawes recorded on 1 July 2024 as "they don't listen to his complaints". What I said on these matters is in my emails; I never said anything hostile about Ms Taylor or any other member of staff. [¶¶ 143–153, 155–181, 266–267, 287–294]
- 8. My health before June 2024.** That before 18 June 2024 I had never been diagnosed with or treated for depression. My general practitioner's record of 16 November 2023 notes poor sleep with shift work, that I could not do shifts without a good sleep, no psychological illness such as depression or psychosis, and mood good. The ADHD and anxiety noted in my history in October 2022, and the psychiatrist referral to Dr Amini renewed on 16 May 2024, the day after the request that I retract my email, listing the medications I was then taking, none of them an antidepressant or an anxiolytic, and for which no appointment could be obtained. [General-practice records disclosed by the Respondent; ¶¶ 76–77]
- 9. Onset, presentation and diagnosis.** That I last worked a shift on 3 June 2024; that on days in May 2024 and in the first week of June 2024 I drove to work, could not bring myself to enter the building, and called in sick from the car park. My symptoms from about 18 June 2024 and their effect on sleep, concentration, self-care and relationships, and on my capacity for work, from being certified unfit from 1 July 2024 to returning on reduced hours and the restrictions recorded in the Checklist of 3 July 2026. Attending Dr Slawinski on 28 June 2024, whose note records "stress at work" and "upset by people not following rules", and Dr Hawes on 1 July 2024, whose note records "work stress", that "they withhold pay at times, no overtime- not processed, manipulate his roster- so he works lates then earlies". Dr Hawes's certificates of 1 July and 8 September 2024 and his psychiatrist referral. My first consultation with Dr Krishnaiah on 24 October 2024, on that referral, when I was told I had major depressive disorder with anxiety state, confirmed by the practice in writing at 11:45 am that day, my fluoxetine increased and quetiapine added at night; his written report of 13 February 2025; and my treatment since. The matters at paragraphs 2 to 7 above, and their effect on me as each occurred and as they accumulated, are the matters I reported to each doctor, as their records show; the events those records name are the facts admitted on 8 September 2026 (Tabs M1 to M4). I offer no clinical or diagnostic opinion.
- 10. After 18 June 2024, and documents.** Meetings scheduled while I was on certified leave; my employment was ended under abandonment provisions with a separation date of 20 September 2024, while I held current certificates, and my later reinstatement; the months without pay after the injury and the claim, with the pay errors never resolved, and the leave since. Documents: rosters and payslips; my emails and messages with my line manager and the Director; the employer's leave and payroll records; and Metro South Health's letter of 5 June 2026.

This outline states the topics on which I will give oral evidence. It is not a statement of evidence and is not verified.

5. Outline of evidence — Mr Cory Harrison-Jones

Direction 2. SERVED on the Respondent. NOT filed. Served in the outlines document, email 2 attachment 4.

OUTLINE OF EVIDENCE – MR CORY HARRISON-JONES

Served pursuant to direction 2 of the Further Directions Order (3) dated 19 August 2026. Not filed in the Industrial Registry.

Mr Harrison-Jones was employed part time at Logan Hospital Switchboard and worked as an operator alongside the Appellant. He was a member of the Switchboard team from 2022, and was still a member on 18 July 2023, when he received the email at item 4 below. He was on shift at the time of the events he describes. He has since left the department and is no longer employed by Metro South Hospital and Health Service; he is now an officer of the Queensland Ambulance Service. He is expected to give evidence of the following.

1. That a function of the Switchboard was to hold and maintain accurate contact and paging numbers for medical staff, including on-call doctors, so that calls and emergency notifications could be directed to the correct person.
2. What he observed, over a period of approximately three months in that time, of the Appellant working to obtain and record updates to medical contact numbers; that the monthly task of updating the spreadsheet of number changes, allocated to a shift, was left undone over that period; that the Appellant placed a reminder in the Communication Book about the allocation of that monthly task; and that he was on shift and saw the Appellant check the contact numbers and write that reminder on handover.
3. That the Communication Book was kept in the operators' room and used by the Switchboard team to communicate and for matters of handover; that he read it in the course of handover during the period he worked at the Switchboard; and that he did not see in it any entry made by the Appellant that criticised any person, directly or indirectly.
4. That he was a recipient of the email sent by Ms Stibbard to the Switchboard team on 18 July 2023 stating that she would be removing everyone's access to the database and that changes were to be sent to her directly.
5. That after access was restricted operators could not correct an entry themselves and had to wait for Ms Stibbard or Ms Taylor to make the change.
6. An occasion on which he was present in the Switchboard room at the commencement of a shift, when the Appellant had arrived and was reading the handover and had not yet signed on: what Ms Taylor said to the Appellant about the removal of a reminder, the sequence in which each of them spoke, and the manner and volume in which it was said, from his own observation.
7. That he did not at any time hear the Appellant say anything hostile about Ms Taylor or about any other member of staff.

This outline is a summary of the evidence the witness is expected to give. It is not a statement of the witness and is not signed by the witness.

6. Outline of evidence — Ms Patricia Conaghan

Direction 2. SERVED on the Respondent. NOT filed. Served in the outlines document, email 2 attachment 4.

OUTLINE OF EVIDENCE – MS PATRICIA CONAGHAN

Served pursuant to direction 2 of the Further Directions Order (3) dated 19 August 2026. Not filed in the Industrial Registry.

Ms Conaghan is an Administration Officer in Switchboard Services at Logan Hospital and has worked in that department alongside the Appellant. She is expected to give evidence of the following.

1. That she and the Appellant each sought an additional two shifts per fortnight, as recorded in his email of 7 August 2023 to Ms Taylor, Ms Reese, Ms Conaghan and Ms Smith.
2. The calls the Switchboard receives and what they are: that a substantial proportion come from people who are distressed, frightened, angry or abusive; that some callers make threats, including threats of harm to the operator, and demand the name of the operator they are speaking to; that calls of that kind are a regular and continuing feature of the work at the console and not occasional events; that the operator on duty is the first person they reach; and that she observed the Appellant receiving such calls.
3. What she observed of clinicians and departments remaining on the line to express their frustration to the operator on duty, and treating the operator as accountable for matters that were the responsibility of others.
4. Occasions she observed of complaints being directed at the Appellant at the console, including an orthopaedic doctor who complained about the Appellant following a change in the way that doctor was to be contacted, and a clinician who made repeated complaints through the Switchboard.
5. The process for relaying urgent pathology results before and after the changes made to the Switchboard process: what the process was, what it became, and what she was told at the console by those calling back about results that had not reached the treating team.
6. What she observed of the Appellant updating pager and contact numbers at the console, and of pager and number updates left unactioned over extended periods.
7. What she was told at the console between 2 and 8 May 2024 by callers reporting that calls had reached the wrong medical team, including calls concerning emergency responses.
8. That during that period operators had no means of correcting an incorrect entry in the directory themselves.
9. That the Communication Book was used by the Switchboard team to communicate and for matters of handover; that she has read it in the course of handover throughout the period she has worked with the Appellant; and that she has not seen in it any entry made by the Appellant that criticised any person, directly or indirectly.
10. That she was not consulted about the change to after-hours on-call arrangements notified by Ms Taylor to the Switchboard on 15 April 2024, and was not asked to vote on that change.
11. That she was not consulted about, and was not balloted in relation to, changes to the Switchboard roster at any time before 30 June 2024.
12. Any change she observed in the Appellant at work over the period, from her own observation. She offers no opinion as to diagnosis or cause.
13. That in the time they have worked together she has not heard the Appellant speak of Ms Taylor, or of any other member of staff, in hostile terms.

This outline is a summary of the evidence the witness is expected to give. It is not a statement of the witness and is not signed by the witness.

7. Schedule of medical documents relied upon, and Tabs M1 to M9

Direction 2, expert limb. SERVED on the Respondent. NOT filed. Email 2, attachment 5. 43 pages, the schedule at page 1.

APPELLANT'S SCHEDULE OF MEDICAL DOCUMENTS RELIED UPON

Purpose and service. Served with the outlines of evidence under direction 2 of the Further Directions Order (3) dated 19 August 2026; not filed. No report has been prepared for the purposes of this proceeding; this schedule and the pages behind it are served as the expert and treating material relied upon. Each document is already held by the Respondent at the item of its amended List of Documents dated 14 August 2026 stated, except item 5, which has not yet been received, and item 6, served herewith.

Use at hearing. Against each document is stated what it is, and is not, relied upon for. Items 3, 4 and 5 contain clinical opinion; the Appellant does not rely upon that opinion otherwise than through the oral evidence of its author. Dr Ravikumar Bangalore Krishnaiah and Dr Peter Hawes are named at items 2 and 3 of the Appellant's list of names of all witnesses filed today, and will be called. Tabs M1 to M4 hold items 1 to 4; Tab M5 the exchange with the author of 5 to 8 September 2026; Tabs M6 to M8 items 5 to 7; Tab M9 the Respondent's responses of 18 February 2026.

The sequence these documents record. No psychological illness before June 2024 (item 1); first presentation attributing symptoms to work, 28 June 2024 (item 1); certification of no capacity from 1 July 2024 (item 2); diagnosis of Major Depressive Disorder with anxiety state, 24 October 2024 (item 3); the treating psychiatrist's account of origin, 13 February 2025 (item 4); continuing effect, 3 July 2026 (item 6).

No.	Document, and where the Respondent holds it	Relied upon for	Not relied upon for
1	General-practice records, Our Medical Ashmore, 1 January 2023 to 1 July 2024 (Respondent's item 11; obtained by the Respondent under the Form 29 signed 4 July 2025).	Prior health: the entry of 16 November 2023 (Dr Nanayakkara) recording poor sleep with shift work, that the Appellant could not do shifts without a good sleep, no psychological illness such as depression or psychosis, and mood good, with melatonin and temazepam prescribed. The referral letter of 16 May 2024 (Dr Zhao), the day after Ms Reese's email of 15 May 2024 at 6:23 pm asking the Appellant to retract his email (facts 76 and 77, admitted), renewing a referral to a psychiatrist, Dr Amini, for "ongoing care and management", which lists among the past-medical-history items "26/10/2022 ADHD" and "26/10/2022 Anxiety", and lists the medications then current, none of which is an antidepressant or an anxiolytic (one entry in that list is redacted as private and unrelated to the injury; it is neither of those things and the Respondent holds the unredacted letter at item 11). First presentation after onset: 28 June 2024 (Dr Slawinski) recording "stress at work" and "upset by people not following rules", reason for visit anxiety; and 1 July 2024 (Dr Hawes) recording "work stress", "been there 5 years", that "they withhold pay at times, no overtime- not processed, manipulate his roster- so he works later then earlier", and "causing anxiety" (facts 185 to 203 and 211 to 221, admitted 8 September 2026). Relied upon as the contemporaneous record of what was reported before any claim decision, dismissal or proceeding.	Entries unrelated to the injury; those are redacted on the extracted pages and marked as such. The entry of 16 November 2023 is relied upon as the contemporaneous record that shift work was affecting the Appellant's sleep, and of what was prescribed for it, before any claim or proceeding (facts 2, 13, 214, 218, 263 to 265 and 269 to 271, admitted 8 September 2026); its clinical significance is a matter for the treating doctors.
2	Work capacity certificates of Dr Peter Hawes dated 1 July, 11 August and 8 September 2024, and of Dr Ki Pang dated 7 August 2024 (Respondent's items 7 and 8).	The stated date of injury, 18 June 2024; that the Appellant was first seen for this injury on 1 July 2024; certification of no functional capacity for any type of work over the period 1 July to 6 October 2024; and the referral to a psychiatrist recorded on 8 September 2024. Review Decision 69983 records at page 17 that the certificate of 1 July 2024 indicated "there was no pre-existing factor or condition", and that "This was maintained in all later work capacity certificates".	The recorded mechanism as a finding of fact. The events are proved by the facts admitted on 8 September 2026, not by the certificates. Nothing as to medication; the medication box is unticked on each.
3	Email from the practice of Dr Ravikumar Bangalore Krishnaiah to the Appellant, 24 October 2024 at 11:45 am, "Medications" (Respondent's item 9; Appellant's List item 22).	That on the morning of 24 October 2024, at the first consultation, the treating psychiatrist told the Appellant, and confirmed in writing at 11:45 am that day, that he was "suffering from psychological injury of Major Depressive Disorder with anxiety state"; that fluoxetine was increased to two capsules "from today" and Seroquel 25 mg commenced at night, "to restore basic needs- sleep, eating and routine". Read with item 2, it records the progression of the injury certified as "anxiety, stress" on 1 July 2024 to a depressive disorder by 24 October 2024. The Appellant's email to QSuper at 5:12 pm that day ("I have finally been able to see a psychiatrist today") is reproduced with it.	The reference to preoccupation with the claim as a cause of the injury. The injury relied upon is that of 18 June 2024; the email records its state four months later.
4	Report of Dr Krishnaiah, Mind and Memory Service, 13 February 2025, prepared for QSuper (Respondent's item 10).	Diagnosis: Major Depressive Disorder with anxious distress (DSM-5 296.23). Severity and functional effect, and treatment (fluoxetine increased to three capsules daily; quetiapine 25 mg at night). The treating clinician's account of origin: "workplace stress stemming from issues with management and rostering"; that the issues "began approximately one year ago when a new manager was appointed"; pay "withheld or delayed for up to five months at a time, leading to significant financial stress"; and that "premature exposure to the workplace is more likely result in significant deterioration". The stressors so recorded correspond to facts admitted by the Respondent on 8 September 2026: management and rostering, and the night-shift line, at facts 211, 212 and 220; the shorter break, at facts 258 to 260; and pay withheld or delayed from the 5 February 2024 fortnight (facts 185 to 190), uncorrected at 13 May (fact 193), "claims older than 3 months" on 28 May (facts 196, 197), a claim effective 30 March "Part Completed" on 30 May (fact 203), and the two February claims absent from the myHR report (fact 210).	Attribution among the individual events at paragraphs 2 to 7 of the Appellant's outline of evidence; any matter after 24 October 2024 as a cause of the injury. The report is marked "not for medico-legal use"; the author's emails of 5 and 8 September 2026 (Tab M5) state of his records "You can use them according to the need to support your legal issues" and identify this report as "the report that captures the relevant information you have requested" on the three matters in issue.
5	Clinical records of Dr Krishnaiah from 24 October 2024 (offered by the practice on 5 September 2026 and requested; not yet received).	What was reported at the first consultation and when; diagnosis and prescribing over time. To be served on receipt. If any direction is required for reliance on them, the Appellant will apply for it.	Nothing until received.
6	Employee Capability Checklist completed by Dr Day Hong Ma, 3 July 2026 (copy served herewith).	Current capacity and restrictions, and the continuing effect of the injury, including "symptom exacerbation on exposure to the identified workplace stressors".	Causation. It post-dates 1 July 2024 and is relied upon for effect and capacity only.
7	Review Decision 69983, reasons dated 24 October 2024, pages 17 and 26 to 27 (Respondent's item 4; contents admitted 18 February 2026).	That the Respondent's own review recorded Dr Hawes's statement to WorkCover of 2 September 2024 that work events were the sole cause (page 17), found "a personal injury of a psychological nature", found employment "was a significant contributing factor" as to factors 2, 3 and 4, and found the March 2024 rostering to be unreasonable management action (facts 257 to 262, admitted).	As a binding determination. The hearing is de novo; the finding is relied upon as an admitted document.

The finding on section 32(1) in the decision under appeal. On the medical evidence then before it — the work capacity certificates at item 2 and Dr Hawes's statement to WorkCover of 2 September 2024 — the Respondent's review of 24 October 2024 found that the Appellant "sustained a personal injury of a psychological nature", and stated: "Having regard to the medical evidence, I am satisfied your employment was a significant contributing factor to the psychological injury" (pages 17 and 26; item 7). The claim was rejected under section 32(5), not section 32(1). The hearing is de novo and that finding does not bind the Commission; it is relied upon as an admitted document.

The Appellant reserves the position as to any further report; if one is to be relied upon, directions will be sought before it is served.



Dated: 9 September 2026
Cory Lea Shepherd, Appellant, self-represented

TAB M1

General-practice records, Our Medical Ashmore, 1 January 2023 to 1 July 2024

Where the Respondent holds it

Respondent's amended List of Documents item 11. Obtained by the Respondent under the Form 29 signed 4 July 2025. The complete record is Exhibit A5; these are extracts of it.

Pages included

Pages 5, 6, 7, 10, 11, 12 and 13 of the 14-page practice export: the consultation of 16 November 2023 (Dr Nanayakkara); the consultations of 16 May 2024 (Dr Zhao, referral renewed) and 28 June 2024 (Dr Slawinski); the consultation of 1 July 2024 (Dr Hawes); the referral letter to Dr Amini of 16 May 2024 with the past medical history; the medical certificate of 28 June 2024; and the work capacity certificate of 1 July 2024.

Pages omitted, and why

Pages 1 to 4 (patient details, medication and prescription lists, consultations of 2023 for unrelated conditions); page 8 (Gold Coast University Hospital discharge letter of 6 June 2024, dental); page 9 (medical certificate of 3 July 2023, unrelated); page 14 (blank). None is relied upon. REDACTIONS: on source pages 5, 6 and 10, entries concerning private medical matters unrelated to the injury have been blacked out by the Appellant and are marked as such on the page. On source page 10 the redaction covers one entry in the list of current medications in the referral letter of 16 May 2024. That entry is a medication for a private condition unrelated to the injury; it is not an antidepressant, an anxiolytic or any other psychotropic medication, and the Respondent can confirm that from its own copy. Nothing else relied upon has been redacted. The Respondent holds the unredacted record at item 11.

Schedule row 1 states what this document is, and is not, relied upon for.

REDACTED by the Appellant: private medical matter unrelated to the injury

Surgery consultation

Recorded by: Dr William Zhao Visit date: 16/05/2024

Recorded on: 16/05/2024

renew referral to psychiatrist

REDACTED by the Appellant: private medical matter unrelated to the injury

Actions:

Letter to Dr Arash Amini printed.

Letter written to Dr Arash Amini re. 1.Specialist Referral Letter Ashmore.

Prescription printed: Melatonin 5mg Tablet, modified release Before bed 6mg / nocte /prn --- Compounded.

REDACTED by the Appellant: private medical matter unrelated to the injury

Surgery consultation

Recorded by: Dr Bogdan Slawinski Visit date: 28/06/2024

Recorded on: 28/06/2024

wants melatonin
to help to sleep

stress at work
upset by people not following rules

Examination:
well, alert, t 36,6

Reason for visit:
Anxiety

Actions:
Prescription printed: Melatonin 5mg Tablet, modified release Before bed 6mg / nocte /prn --- Compounded.
Letter printed.
Letter written re. 1. Medical Certificate Ashmore.

Surgery consultation

Recorded by: Dr Peter Hawes Visit date: 01/07/2024
Recorded on: 01/07/2024

work problems- ethical complaint about manager and director, they don't follow the law
been there 5 years
feels he is undervalued
they withhold pay at times, no overtime- not processed, manipulate his roster- so he works lates then earlies, not
allowed to do this
they don't listen to his complaints
all this is stressing him out, causing anxiety,

Reason for visit:
work stress

Actions:
Letter printed.
Letter written re. QLD Work capacity certificate.

Investigations:



Our Medical Ashmore

566 Olsen Avenue, Ashmore QLD 4214

HealthLink ourmedic | **Phone** 07 5647 5000

Fax 07 5616 8014

Email reception@ourmedicalashmore.com.au

A Cornerstone Health Pty Ltd Medical centre

Operated by

Wycombe Services Pty Ltd

ABN 40 613103 450

www.ourmedicalhome.com.au

16/05/2024

Dr Arash Amini

Evandale Practice

Suite 2, 1st Floor Evandale Place, 142 Bundall Road

Bundall 4217

Phone: (07) 5510 3122

Fax: (07) 5510 3144

Re: Mr Cory Lea Shepherd

Add: U207 W\l r\ n1 Marina Drive

Benowa 4217

DOB: 11/01/1991

Medicare No: 4330920095

Ph: 0422438627

Dear Arash,

Thank you for seeing Mr Cory Lea Shepherd for ongoing care and management.

Current medications are:

Azithromycin 500mg Tablet

2 Tablets stat.

Melatonin 5mg Tablet, modified release

Before bed 6mg / nocte / prn --- Compounded.

REDACTED by the Appellant: private medical matter unrelated to the injury

Past Medical History:

26/10/2022	ADHD
26/10/2022	Anxiety
28/06/2023	Infection

Allergies:

Nil known.

Yours faithfully,

Dr William Zhao
BMED,AMC, FRACGP
Provider No: 264389EW



Our Medical Ashmore

566 Olsen Avenue, Ashmore QLD 4214

HealthLink ourmedic | Phone 07 5647 5000

Fax 07 5616 8014

Email reception@ourmedicalashmore.com.au

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Operated by

Wycombe Services Pty Ltd

ABN 40 613103 450

www.ourmedicalhome.com.au

MEDICAL CERTIFICATE

28/06/2024

THIS IS TO CERTIFY THAT :

Mr Cory Lea Shepherd has a medical condition and will be unfit for Work/ Study/ Usual activity
from 27/06/2024 to 11/07/2024 inclusive.

Dr Bogdan Slawinski
General Practitioner, MBBS, FACEM
208300QJ

Form 132M — Version 1

IMPORTANT INFORMATION: Work is an important part of recovery. In most cases an early return to work (or remaining at work) is beneficial for health and wellbeing. The treating practitioner's guidance increases the likelihood of positive return to work outcomes. A worker receiving continued support is three times more likely to regain their capacity to work. Consider the health benefits of work when certifying the patient's capacity.

Name Cory Shepherd Date of Birth 11/01/1991
Mobile number 0422438627 Claim number S23LW142013 ☒ New claim ☐ Claim is report only
Occupation (if known) Admin worker Patient's employer Qld Health

Date of examination	01/07/2024	Patient's stated date of injury	18/06/2024	Patient was first seen at this practice/hospital for this injury/disease on	01/07/2024
---------------------	------------	---------------------------------	------------	---	------------

The patient is/was suffering from (List all work-related diagnoses. If symptoms only, tick "Provisional diagnosis") ☐ Provisional diagnosis

Patient's stated mechanism of injury harrassment at work

Is this consistent with your clinical findings?

☒ Yes ☐ Unclear

Describe mechanism in detail ongoing breaking of workplace rules by bosses, victimizing him

Pre-existing factors or condition

aggravated (if not previously supplied)

Patient requires/d treatment from 01/07/2024 to 04/08/2024 to be reviewed again on 04/08/2024 ☐ No further review

Treatment needs time way from this environment

I have prescribed medication that may impede safe work, travel or cognitive function ☐ No ☐ Yes

Referrals ☐ Diagnostic ☐ Allied Health ☐ ☐ Name/discipline ☐ Details (*specify*)

Specialist/GP

☐ The certified injury does not prevent a return to pre-injury duties. Do not complete part E. Go to Part F. ☐ If suitable duties available, can return to some form of work from ☒ No functional capacity for any type of work until

Complete the below section if you certified no functional capacity for any type of work

If no functional capacity, state why? (if no capacity for more than 7 days, the insurer may contact you to obtain more information)

stressed and anxious.

Estimated time to return to
some form of work duties
unknown

Estimated time to return to full duties
unknown

(Optional for emergency medical practitioners/dental practitioners. Nurse practitioners not to complete.) No change since last certificate

Certification should be based on what CAN be done, NOT available duties. Consider what the patient can do, either at work or home.

Function/task (<i>patient's usual functional ability</i>)	Is functional ability affected by injury/condition?	What patient can do (if "yes" is selected)
	No Yes Note any restrictions (<i>if relevant</i>)	
Walking		
Staircase		
Transfer from bed to chair		
Transfer from wheelchair to toilet		
Transfer from wheelchair to car		
Transfer from car to wheelchair		
Transfer from wheelchair to bed		
Transfer from wheelchair to shower		
Transfer from wheelchair to bath		
Transfer from wheelchair to toilet		
Transfer from wheelchair to kitchen		
Transfer from wheelchair to living room		
Transfer from wheelchair to bedroom		
Transfer from wheelchair to bathroom		
Transfer from wheelchair to garden		
Transfer from wheelchair to parking area		
Transfer from wheelchair to street		
Transfer from wheelchair to public transport		
Transfer from wheelchair to shop		
Transfer from wheelchair to restaurant		
Transfer from wheelchair to cinema		
Transfer from wheelchair to theatre		
Transfer from wheelchair to museum		
Transfer from wheelchair to library		
Transfer from wheelchair to post office		
Transfer from wheelchair to bank		
Transfer from wheelchair to police station		
Transfer from wheelchair to fire station		
Transfer from wheelchair to hospital		
Transfer from wheelchair to court		
Transfer from wheelchair to prison		
Transfer from wheelchair to airport		
Transfer from wheelchair to train station		
Transfer from wheelchair to bus stop		
Transfer from wheelchair to taxi rank		
Transfer from wheelchair to car park		
Transfer from wheelchair to parking space		
Transfer from wheelchair to road		
Transfer from wheelchair to footpath		
Transfer from wheelchair to cycle path		
Transfer from wheelchair to pedestrian crossing		
Transfer from wheelchair to zebra crossing		
Transfer from wheelchair to roundabout		
Transfer from wheelchair to junction		
Transfer from wheelchair to intersection		
Transfer from wheelchair to roundabout exit		
Transfer from wheelchair to roundabout entrance		
Transfer from wheelchair to roundabout middle		
Transfer from wheelchair to roundabout left		
Transfer from wheelchair to roundabout right		
Transfer from wheelchair to roundabout straight		
Transfer from wheelchair to roundabout back		
Transfer from wheelchair to roundabout front		
Transfer from wheelchair to roundabout side		
Transfer from wheelchair to roundabout end		
Transfer from wheelchair to roundabout start		
Transfer from wheelchair to roundabout finish		
Transfer from wheelchair to roundabout turn		
Transfer from wheelchair to roundabout lane		
Transfer from wheelchair to roundabout slip		
Transfer from wheelchair to roundabout exit slip		
Transfer from wheelchair to roundabout exit slip lane		
Transfer from wheelchair to roundabout exit slip lane end		
Transfer from wheelchair to roundabout exit slip lane start		
Transfer from wheelchair to roundabout exit slip lane middle		
Transfer from wheelchair to roundabout exit slip lane left		
Transfer from wheelchair to roundabout exit slip lane right		
Transfer from wheelchair to roundabout exit slip lane straight		
Transfer from wheelchair to roundabout exit slip lane back		
Transfer from wheelchair to roundabout exit slip lane front		
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Transfer from wheelchair to roundabout exit slip lane straight		
Transfer from wheelchair to roundabout exit slip lane back		
Transfer from wheelchair to roundabout exit slip lane front		
Transfer from wheelchair to roundabout exit slip lane side		
Transfer from wheelchair to roundabout exit slip lane end		
Transfer from wheelchair to roundabout exit slip lane start		
Transfer from wheelchair to roundabout exit slip lane middle		
Transfer		

Lower limb
Upper limb
Hand function
Spinal function
Cognition/psychosocial functioning
Driving a car
Operating machinery/heavy vehicle
Manual tasks
Other

Is functional ability affected by injury/condition?
No **Yes** Note any restrictions (if relevant)

What patient can do (if "yes" is selected)

(Optional for emergency medical practitioners/dental practitioners. Nurse practitioners not to complete.)

What workplace modifications are required to facilitate return to work? (e.g. work site assessment, psychosocial considerations)

Other considerations or factors that may affect recovery (the insurer can arrange appropriate support)

☐ I require a suitable duties program to be provided to me for approval

I have discussed injury requirements and return to work with the patient and ☐ Employer ☐ Insurer ☐ Rehabilitation provider

I have discussed the information contained in this certificate with the patient. I have provided the clinical information in this certificate.

Name Peter Hawes

Practice/hospital Our Medical Ashmore

Postal Address

566 Olsen Avenue, Ashmore

QLD 4214

Email admin@ourmedicalashmore.com.au

Phone 0756475000 Date 01/07/2024

Signature _____

Date 01/07/2024

DATE 07/07/2024

Further information www.worksafe.qld.gov.au/medicalsupport

All enquiries (medical/dental/nurse practitioner, patient, employer) 1300 362 128

Under the Workers' Compensation and Rehabilitation Act 2003 (the Act), the workers' compensation insurer is authorised to collect the information on this form to process the claimant's application for compensation. The information contained in this form may be disclosed to the claimant's employer, another insurer, medical or allied health providers or any other workers' compensation authority in any jurisdiction. The claimant may be contacted by the insurer, and the insurer may contact the claimant's employer and any other medical, allied health or rehabilitation provider about the injury. This form was approved by the Workers' Compensation Regulator on 31 May 2016, pursuant to section 16/5539

586 of the Act. © State of Queensland (Queensland Treasury) 2016



TAB M2

Work capacity certificates: Dr Ki Pang, 7 August 2024; Dr Peter Hawes, 11 August 2024 and 8 September 2024

Where the Respondent holds it

Respondent's amended List of Documents items 7 (Hawes, 1 July, 11 August and 8 September 2024) and 8 (Pang, 7 August 2024).

Pages included

The certificates of 7 August 2024 (Dr Pang), 11 August 2024 and 8 September 2024 (Dr Hawes), reproduced from the scanned copies annexed to the Appellant's application in TD/2024/110 filed 25 October 2024. Together with the certificate of 1 July 2024 at Tab M1 (bundle page 9), they certify no capacity for any work over the period 1 July to 6 October 2024, each recording the stated date of injury as 18 June 2024 and first presentation on 1 July 2024; the certificate of 8 September 2024 records the referral to a psychiatrist.

Pages omitted, and why

None. The medication box on each certificate is unticked; nothing is relied upon as to medication from these certificates.

Schedule row 2 states what this document is, and is not, relied upon for.

Work capacity certificate - workers' compensation

Workers' Compensation and Rehabilitation Act 2003

IMPORTANT INFORMATION Work is an important part of recovery. In most cases, practitioner's guidance increases the likelihood of positive return to work outcomes. Consider the health benefits of work when certifying the patient's capacity.

early return to work (or remaining at work) is beneficial for health and wellbeing. The treating worker receiving continued support is three times more likely to regain their capacity to work.

Part A - Patient details

Name Cory Shepherd

Mobile number 0422438627

Occupation (if known) Admin worker

Claim number S23LW14203

Date of Birth 11/01/1991

☒ New claim

Patient's employer Qld Health

☐ Claim is report only

Part B - Injury details

Date of examination 01/07/2024

Patient's stated date of injury 18/06/2024

Patient was first seen at this practice/hospital for this injury/disease on

01/07/2024

The patient is/was suffering from (List all work-related diagnoses, if symptoms only, tick "Provisional diagnosis") ☐ Provisional diagnosis

Patient's stated mechanism of injury harrasment at work

anxiety, stress

Is this consistent with your clinical findings?
☒ Yes ☐ Unclear

Describe mechanism in detail

Pre-existing factors or condition

aggravated (if not previously supplied)

ongoing breaking of workplace rules by bosses, victimizing him

Part C - Treatment plan

Patient requires/d treatment from 01/07/2024 to 09/08/2024

to be reviewed again on 09/08/2024

☐ No further review

Treatment needs time away from this environment

I have prescribed medication that may impede safe work, travel or cognitive function ☐ No ☐ Yes

Referrals ☐ Diagnostic ☐ Allied Health ☐ Specialist/GP

Name/discipline

Details (specify)

Part D - Capacity for work (choose from one of three options)

☐ The certified injury does not prevent a return to pre-injury duties. Do not complete part E. Go to Part F.

☐ If suitable duties available, can return to some form of work from

☒ No functional capacity for any type of work until

Complete the below section if you certified no functional capacity for any type of work

If no functional capacity, state why? (if no capacity for more than 7 days, the insurer may contact you to obtain more information)

stressed and anxious,

Estimated time to return to some form of work duties unknown

Estimated time to return to full duties unknown

Part E - Functional ability

(Optional for emergency medical practitioners/dental practitioners. Nurse practitioners not to complete.)

No change since last certificate

Certification should be based on what CAN be done, NOT available duties. Consider what the patient can do, either at work or home.

Function/task (patient's usual functional ability)

Is functional ability affected by injury/condition?

No Yes Note any restrictions (if relevant)

What patient can do (if "yes" is selected)

Lower limb

No

Upper limb

No

Hand function

No

Spinal function

No

Cognition/psychosocial functioning

No

Driving a car

No

Operating machinery/heavy vehicle

No

Manual tasks

No

Other

No

Part F - Rehabilitation at work - return to work plan

(Optional for emergency medical practitioners/dental practitioners. Nurse practitioners not to complete.)

What workplace modifications are required to facilitate return to work? (e.g. work site assessment, psychosocial considerations)

Other considerations or factors that may affect recovery (the insurer can arrange appropriate support)

☐ I require a suitable duties program to be provided to me for approval

I have discussed injury requirements and return to work with the patient and

☐ Employer

☐ Insurer

☐ Rehabilitation provider

Part G - Medical/dental/nurse practitioner details and statement (or use practice/hospital stamp)

I have discussed the information contained in this certificate with the patient. I have provided the clinical information in this certificate.

Name Ki Pang

Practice/hospital Our Medical Ashmore

Postal Address

588 Olsen Avenue, Ashmore, QLD 4214

Further information www.worksafe.qld.gov.au/medicalsupport

Enquiries (medical/dental/nurse practitioner, patient, employer) 130 362 121

Under the Workers' Compensation and Rehabilitation Act 2003 (the Act), the workers' compensation claimant's application for compensation (the information contained in this form may be disclosed to the claimant or any other workers' compensation authority or to the insurer or the insurer's agent, or any other medical, allied health or rehabilitation provider, about the injury. This form was approved by the Workers' Compensation and Rehabilitation Board on 31 May 2016.

686 of the Act © State of Queensland (Queensland Treasury) 2016

Work capacity certificate - workers' compensation

Form 132M - Version 1

Workers' Compensation and Rehabilitation Act 2003

IMPORTANT INFORMATION: Work is an important part of recovery. In most cases an early return to work (or remaining at work) is beneficial for health and wellbeing. The treating practitioner's guidance increases the likelihood of positive return to work outcomes. A worker receiving continued support is three times more likely to regain their capacity to work.

Consider the health benefits of work when certifying the patient's capacity.

Part A - Patient details

Name: Cory Shepherd Date of Birth: 11/01/1991
Mobile number: 0422438627 Claim number: S23LW142013 ☐ New claim ☐ Claim is report only
Occupation (if known): Admin worker Patient's employer: Qld Health

Part B - Injury details

Date of examination: 11/08/2024 Patient's stated date of injury: 18/06/2024 Patient was first seen at this practice/hospital for this injury/disease on: 01/07/2024

The patient is/was suffering from (List all work-related diagnoses): anxiety, stress

If symptoms only, tick "Provisional diagnosis" ☐ Provisional diagnosis

Patient's stated mechanism of injury: harrasment at work

Is this consistent with your clinical findings?

☒ Yes ☐ Unclear

Describe mechanism in detail: ongoing breaking of workplace rules by bosses, victimizing him

Pre-existing factors or condition

aggravated (if not previously supplied)

Part C - Treatment plan

Patient requires/d treatment from: 11/08/2024 to: 08/09/2024 to be reviewed again on: 08/09/2024 ☐ No further review

Treatment needs time away from this environment

I have prescribed medication that may impede safe work, travel or cognitive function ☐ No ☐ Yes

Referrals ☐ Diagnostic ☐ Allied Health ☐ Name/discipline: Details (specify)

Specialist/GP

Part D - Capacity for work (choose from one of three options)

☐ The certified injury does not prevent a return to pre-injury duties. Do not complete Part E. Go to Part F. ☐ If suitable duties available, can return to some form of work from ☒ No functional capacity for any type of work until 8/9/24

Complete the below section if you certified no functional capacity for any type of work

If no functional capacity, state why? (if no capacity for more than 7 days, the insurer may contact you to obtain more information)

stressed and anxious,

Estimated time to return to some form of work duties: unknown

Estimated time to return to full duties: unknown

Part E - Functional ability

(Optional for emergency medical practitioners/dental practitioners. Nurse practitioners not to complete.) No change since last certificate

Certification should be based on what CAN be done, NOT available duties. Consider what the patient can do, either at work or home.

Function/task (patient's usual functional ability)	Is functional ability affected by injury/condition?	What patient can do (if "yes" is selected)
	No Yes Note any restrictions (if relevant)	
Lower limb	No	
Upper limb	No	
Hand function	No	
Spinal function	No	
Cognition/psychosocial functioning	No	
Driving a car	No	
Operating machinery/heavy vehicle	No	
Manual tasks	No	
Other	No	

Part F - Rehabilitation at work - return to work plan

(Optional for emergency medical practitioners/dental practitioners. Nurse practitioners not to complete.)

What workplace modifications are required to facilitate return to work? (e.g. work site assessment, psychosocial considerations)

Other considerations or factors that may affect recovery (the insurer can arrange appropriate support)

I require a suitable duties program to be provided to me for approval
have discussed injury requirements and return to work with the patient and ☐ Employer ☐ Insurer ☐ Rehabilitation provider

Part G - Medical/dental/nurse practitioner details and statement (or use practice/hospital stamp)

I have discussed the information contained in this certificate with the patient. I have provided the clinical information in this certificate.

Name: Peter Hawes
Practice/hospital: Our Medical Ashmore
Postal Address:
16 Olsen Avenue, Ashmore
QLD 4214

Email: admin@ourmedicalashmore.com.au
Phone: 0756475000 Date: 11/08/2024
Signature: [Signature]

Further information: www.worksafe.qld.gov.au/medicalsupport
enquiries (medical/dental/nurse practitioner, patient, employer) 1300 362 128

Under the Workers' Compensation and Rehabilitation Act 2003 (the Act), the workers' compensation insurer is authorised to collect the information on this form to process the claimant's application for compensation. The information contained in this form may be disclosed to the claimant's employer, another insurer, medical or allied health providers or any other workers' compensation authority in any jurisdiction. The claimant may be contacted by the insurer, and the insurer may contact the claimant's employer and any other medical, allied health or rehabilitation provider about the injury. This form was approved by the Workers' Compensation Regulator on 31 May 2018, pursuant to section 585 of the Act. © State of Queensland (Queensland Treasury) 2016



Work capacity certificate – workers' compensation

Workers' Compensation and Rehabilitation Act 2003

IMPORTANT INFORMATION: Work is an important part of recovery. In most cases an early return to work (or remaining at work) is beneficial for health and well-being. Practitioner's guidance increases the likelihood of positive return to work outcomes. A worker receiving continued support is three times more likely to regain their capacity. Consider the health benefits of work when certifying the patient's capacity.

Part A - Patient details

Name: Cory Shepherd
 Mobile number: 0422438627
 Occupation (if known): Admin worker
 Claim number: S33LW142013
 Date of Birth: 11/01/1991
☐ New claim
☒ Claim is report

Part B - Injury details

Date of examination: 11/08/2024
 Patient's stated date of injury: 18/06/2024
 Patient was first seen at this practice/hospital for this injury/disease on: 01/07/2024

The patient is/was suffering from (List all work-related diagnoses. If symptoms only, tick "Provisional diagnosis")
☐ Provisional diagnosis

Patient's stated mechanism of injury: harassment at work

anxiety, stress

Is this consistent with your clinical findings?
☒ Yes ☐ Unclear

Describe mechanism in detail:
 Pre-existing factors or condition:

ongoing breaking of workplace rules by bosses, victimizing him

aggravated (if not previously supplied)

Part C - Treatment plan

Patient requires/d treatment from: 08/09/2024 to 06/10/2024 to be reviewed again on: 06/10/2024
☐ No further review

Treatment needs time away from this environment
 I have prescribed medication that may impede safe work, travel or cognitive function: ☐ No ☐ Yes

Referrals: ☒ Diagnostic ☐ Allied Health ☐ Name/discipline: Psychiatrist
 Specialist/GP: Details (specify):

Part D - Capacity for work (choose from one of three options)

☐ The certified injury does not prevent a return to pre-injury duties. Do not complete part E. Go to Part F.
☐ If suitable duties available, can return to some form of work from:
☒ No functional capacity for any type of work until:

Complete the below section if you certified no functional capacity for any type of work
 If no functional capacity, state why? (if no capacity for more than 7 days, the insurer may contact you to obtain more information)

Estimated time to return to some form of work duties: unknown
 Estimated time to return to full duties: unknown

Part E - Functional ability

(Optional for emergency medical practitioners/dental practitioners. Nurse practitioners not to complete.) No change since last certificate
 Certification should be based on what CAN be done, NOT available duties. Consider what the patient can do, either at work or home.

Function/task (patient's usual functional ability)	Is functional ability affected by injury/condition?	What patient can do (if "yes" is selected)
Lower limb	No Yes ☐ No ☒	
Upper limb	No Yes ☐ No ☒	
Hand function	No Yes ☐ No ☒	
Spinal function	No Yes ☐ No ☒	
Cognition/psychosocial functioning	No Yes ☐ No ☒	
Driving a car	No Yes ☐ No ☒	
Operating machinery/heavy vehicle	No Yes ☐ No ☒	
Manual tasks	No Yes ☐ No ☒	
Other	No Yes ☐ No ☒	

Part F - Rehabilitation at work - return to work plan

(Optional for emergency medical practitioners/dental practitioners. Nurse practitioners not to complete.)
 What workplace modifications are required to facilitate return to work? (e.g. work site assessment, psychosocial considerations)

Other considerations or factors that may affect recovery (the insurer can arrange appropriate support)

I require a suitable duties program to be provided to me for approval
 We discussed injury requirements and return to work with the patient and:

☐ Employer ☐ Insurer ☐ Rehabilitation

G - Medical/dental/nurse practitioner details and statement (or use practice/hospital stamp)

discussed the information contained in this certificate with the patient. I have provided the clinical information in this certificate.
 Peter Hawes
 Email: admin@ourmedicalashmore.com.au
 Phone: 0756475000
 Date: 08/09/2024
 Signature: [Signature]

Information: www.worksafe.qld.gov.au/medicalsupport
 (medical/dental/nurse practitioner, patient, employer) 1300 362 128

Workers' Compensation and Rehabilitation Act 2003 (the Act) the workers' compensation insurer is authorised to collect the information on this form to process the claim for compensation. The information contained in this form may be disclosed to the claimant's employer, another insurer, medical or allied health providers, workers' compensation authority in any jurisdiction. The claimant may be contacted by the insurer, and the insurer may contact the claimant's employer and any medical or allied health or rehabilitation provider about the injury. This form was approved by the Workers' Compensation Regulator on 31 May 2016, pursuant to section 150 of the Act.
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TAB M3

Email from the practice of Dr Ravikumar Bangalore Krishnaiah to the Appellant, 24 October 2024 at 11:45 am, "Medications"; and the Appellant's email to QSuper at 5:12 pm the same day

Where the Respondent holds it

The practice email: Respondent's amended List of Documents item 9; Appellant's List item 22. The QSuper email: from the QSuper income-protection correspondence bundle held by the Appellant; served herewith.

Pages included

Page 1 of the practice email, as forwarded from the Appellant's mailbox on 25 November 2024 and bearing the original header of 24 October 2024 11:45 am. Then the page of the QSuper correspondence bundle carrying the Appellant's email of 24 October 2024 at 5:12 pm ("I have finally been able to see a psychiatrist today"), which fixes the date of first consultation from a second, contemporaneous document.

Pages omitted, and why

Page 2 of the practice email (the continuation of the sender's signature block and a list of helpline numbers). The remainder of the QSuper bundle. Neither is relied upon.

Schedule row 3 states what this document is, and is not, relied upon for.

Psychiatrist Fw: Medications

From: Cory Shepherd

coryshepherd1@hotmail.com

To: Zach Joubert zjoubert@stonegroup.com.au

Sent: Monday 25 November 2024 at 13:36

Get [Outlook for iOS](#)

From: Reception

[<Reception@mindandmemoryservice.com.au>](mailto:Reception@mindandmemoryservice.com.au)

Sent: Thursday, October 24, 2024 11:45 am

To: coryshepherd1@hotmail.com

[<coryshepherd1@hotmail.com>](mailto:coryshepherd1@hotmail.com)

Subject: Medications

Hi Cory,

As mentioned, you are suffering from psychological injury of Major Depressive Disorder with anxiety state and obsessive preoccupation about work cover claim and perceived injustice to an extent that your selfcare and daily activities have been neglected.

Please increase Fluoxetine to 2 capsules from today and take Seroquel 25 mg half a tablet for 3 nights and increase to 1 tablet at night

Both these medications helps you to restore basic needs- sleep, eating and routine as well as reduction in obsessive rumination and preoccupation.

I will see you in 3-4 weeks time

Kind regards

Ravi

Dr Ravikumar Bangalore Krishnaiah

Thank you.

Get [Outlook for iOS](#)

From: Louise Ings <Louise.Ings@art.com.au>
Sent: Friday, October 25, 2024 7:59 am
To: Cory Shepherd <coryshepherd1@hotmail.com>
Subject: RE: Party:195438787 Claim:CLM-317073

Hi Cory,

Thank you for sending through the certificate. Great to hear you had your initial appointment with the psychiatrist.

I am continuing to review your pending claim. I do understand it is a stressful time. I do have service level agreements and am within timeframes to make a claim decision. I am on leave today and will continue to assess your claim when I return on Monday. I will update you early next week.

Many thanks and kind regards,

Louise Ings
Claims Manager
225 George Street
Sydney, NSW, 2000
P 1300 360 750 | F 07 3239 1139
Direct P 07 3156 6764 | W qsuper.qld.gov.au



We acknowledge the Traditional Custodians of the lands where we live and work, and their connections to land, sea and community.

From: Cory Shepherd <coryshepherd1@hotmail.com>
Sent: Thursday, 24 October 2024 5:12 PM
To: Louise Ings <Louise.Ings@art.com.au>
Subject: Re: Party:195438787 Claim:CLM-317073

Hi Louise,

I have finally been able to see a psychiatrist today.

I have attached a medical certificate from them if you need it.

I also would like an update on the qsuper income protection if that is possible. I am experiencing a lot of financial distress.

Additionally, I am unable to make other applications with Q super for financial distress.

I am only able to make an application under the financial condition of receiving months of Centrelink payments which I have not.

Kind regards,

TAB M4

Report of Dr Krishnaiah, Mind and Memory Service, 13 February 2025, prepared for QSuper

Where the Respondent holds it

Respondent's amended List of Documents item 10.

Pages included

All four pages. The report is reproduced whole so that nothing relied upon is read out of its context.

Pages omitted, and why

None.

Schedule row 4 states what this document is, and is not, relied upon for.

This information was disclosed for Qsuper and not for medico-legal use. If a patient or any third party requests a copy, please direct them to our office. This message contains confidential information and is intended only for the individual named. If you are not the named addressee, you should not disseminate, distribute or copy this document.

Please notify the sender immediately if you have received this document by mistake and delete that from your system.

Louise Ings
Claims Manager

13-Feb-2025

RE: Report for Mr Cory Shepherd (11-Jan-1991)

104/158 Scarborough Street
Southport QLD 4215 0422 438 627

Many thanks for reaching out for the report. I have undertaken clinical assessment, reviewed materials presented to me regarding his unfair dismissal, workplace conflicts, alleged bullying, harassment and Workcover matter. Further obtained collateral information from his mother independently. Based on above, I am able to provide information on his mental health symptoms, working diagnosis and opinion on impact of his mental health on his daily functioning and occupational impairment. **Mr Cory Shepherd suffers from Major Depressive Disorder with anxious distress (DSM 5- 296.23).**

Mr Cory Shepherd presented with workplace stress stemming from issues with management and rostering at Queensland Health. He reported financial difficulties due to withheld pay, relationship strain, sleep disturbances, and mood changes with difficulty concentrating. These issues began approximately one year ago when a new manager was appointed. After raising concerns about rostering, fatigue management, and workplace policies, he experienced changes in work conditions including shorter breaks, more night shifts, and reduced penalty rates. His pay was withheld or delayed for up to five months at a time, leading to significant financial stress and strain on his relationship.

Cory presented as articulate and engaged during the consultation. Appeared physically drained and showed signs of neglecting personal hygiene, reporting a decrease in his usual self-care routine. He was anxious throughout the interview with intermittent intense sweating, shaking and hesitant speech. Subjectively he described mood as low, rating it 3/10. Affect was observed to be restricted with anxious distress. Expressed feelings of worthlessness and dehumanisation related to his current employment situation and financial stress. Thought Form and Content: Thought form was coherent and goal-directed. Thought content was preoccupied with workplace issues, including perceived unfair treatment, financial stress due to withheld pay, and the ongoing legal proceedings. Demonstrated rumination about these issues, which appeared to dominate his cognitive processes. There was no evidence of perceptual disturbances or hallucinations reported or observed during the consultation. He was oriented to time, place, and person, however was not able to sustain focus on conversation, kept repeating the same issues despite gentle nudging to relevant questions, evident of obsessive nature to his thoughts affecting his cognitive functions. He reported difficulties with concentration and literacy during periods of high stress at work. No formal cognitive assessment was conducted during this consultation. Cory demonstrated partial insight into his current mental state and the impact of workplace stress on his overall functioning. Judgement appeared impaired due to his mental state.

Risk Assessment: No current suicidal ideation or plan reported. Protective factors include a supportive partner and willingness to seek help. However, ongoing financial and occupational stressors may potentially increase risk in the future. There was no current suicidal ideation reported, and no evidence of perceptual disturbances or thought disorder. Insight was assessed as partial.

The severity of his symptoms has significantly impacted his occupational functioning, leading to his current unemployment. Social functioning has been impaired, particularly in his relationship, and he reported reduced self-care with a decreased hygiene routine. Cognitive difficulties, particularly with concentration and working memory.

This information was disclosed for the only reason of clinical information and not for medico-legal use. If a patient or any third party requests a copy, please direct them to our office. This message contains confidential information and is intended only for the individual named. If you are not the named addressee, you should not disseminate, distribute or copy this document. Please notify the sender immediately if you have received this document by mistake and delete that from your system.

The treatment plan includes continuing fluoxetine 20mg daily with monitoring for side effects and efficacy. Engagement in self-care activities and stress management techniques has been encouraged, and a referral for psychological therapy (CBT) is being considered. He started treatment with Fluoxetine which was increased to 3 capsules per day and night-time Quetiapine. Consideration was given to adding a low-dose antipsychotic-quetiapine 25mg nocte, for rumination and sleep disturbances.

1. Please detail the impact of the condition on ability to engage in daily activities (such as self-care, domestic activities, driving and community engagement).

Cory's condition has significantly impacted overall daily functioning, with notable effects on occupational performance, financial stability, personal relationships, and self-care routines. The stress and anxiety related to the workplace issues appear to have permeated multiple areas of Cory's life, leading to a marked decrease in his quality of life and ability to engage in normal daily activities.

Cory's workplace stress and related issues have significantly affected various aspects of his daily functioning:

- a. Occupational functioning: Currently not working due to ongoing workplace issues and disputes.
- b. financial management: Experiencing financial difficulties due to withheld or delayed pay, impacting overall quality of life.
- c. Relationships: Strain in current relationship due to financial stress, with partner feeling like a "sugar mommy".
- d. Self-care: Reported decline in general hygiene routines, no longer maintaining daily shower habits as previously done. He has been experiencing sleep disturbances, initial insomnia, obsessive rumination, fear driven thoughts, extensive researching, and planning on fact findings for work-related issues including reading up on policies, procedures, and legislations. His concentration was impaired, reported becoming "illiterate" due to stress, affecting ability to communicate effectively. He has been feeling flat in his mood, varied with unexplained irritability, self-directed anger, and feelings of worthlessness. He stopped spending time with his partner, friends and became socially withdrawn. He was unable to engage in previous interests and activities, suggesting a loss of enjoyment in previously pleasurable pursuits. He stopped exercising and has been neglecting his nutritional needs. His self-confidence and self-esteem had been low to an extent that he started to doubt his own values, beliefs, and ability to make his own decision.

2. Are there any other non-medical factors influencing capacity to work? If yes, please provide brief details.

Yes, there are several non-medical factors influencing capacity to work. Firstly, financial stress due to withheld or delayed pay for up to 5 months at a time has significantly impacted overall wellbeing and ability to meet basic needs. This has led to relationship strain and feelings of worthlessness. Ongoing workplace dispute and legal proceedings, including WorkCover claim and industrial relations review, have consumed his whole life severely affecting his functioning. The prolonged nature of these processes is contributing to rumination and preoccupation with workplace issues. His loss of routine and daily structure had changed since stopping work, disrupted normal coping mechanisms and self-care practices. This has led to a decline in personal hygiene habits and engagement in previously enjoyed activities. His perceived discrimination and unfair treatment in the workplace have eroded self-esteem and confidence in professional abilities. This may impact future job-seeking behaviours and workplace interactions. Cory had limited support from his union representatives has left feeling isolated and unsupported in navigating complex workplace issues, potentially exacerbating stress, and anxiety.

3. What is the current treatment plan and effectiveness of this treatment? Include type of

treatment, frequency, duration, starting/ending dates or ongoing (timeframes).

Current Treatment Plan:

Pharmacological Medications:

1. Fluoxetine 20mg daily, gradually increased to 3 in the morning, response has been gradual in nature.
2. Addition of low-dose antipsychotic -quetiapine 25mg nocte for rumination and sleep, helped him to switch off from the obsessive thoughts

3. Vyvanse 30 mg once a day started recently to manage his treatment resistant depressive state and cognitive impairment

Psychological treatment:

Cory's condition would benefit from regular psychological therapy (CBT) sessions and trauma focused interventions. I will be exploring his willingness to engage in therapy sessions.

Regular follow-up review scheduled to monitor progress, review mental state, and optimise treatment.

4. Please detail how the claimed condition impacts capacity to engage in the duties required of their own occupation:

a. Current capacity for work:

In my opinion Mr Shepherd has complete incapacity to undertake duties required for his own occupation.

b. Current barriers for returning to work within their own occupation:

Ongoing symptoms of severe nature of Major depressive Disorder and significant functional decline

c. Recommendations or support that can be made to assist in returning to work:

- Continue current medications review and optimisation required based on the progress
- Engage in psychological therapy (CBT) to manage symptoms and develop coping strategies
- Implement stress management techniques and self-care activities
- Regular psychiatric follow-up to monitor progress and adjust treatment as needed

d. Long-term barriers:

The duration of these barriers is unclear at this stage. Ongoing treatment and support may help address the current issues, but the outcome of the workplace dispute and legal proceedings could potentially have long-term impacts on the individual's ability to return to their previous occupation. Further, focusing on recovery is key to build psychological capacity to return to work with appropriate focused interventions to determine work-readiness.

5. Please briefly outline current Return to Work goal. If unclear, please provide an indication of a suitable time to review.

Given the complex workplace issues, ongoing WorkCover claim and disputed unfair dismissal proceedings and impaired decision-making abilities it is unclear at this stage the work goal and return to work. I will further update on this after next consultation. Please note that premature exposure to the workplace is more likely result in significant deterioration in his mental health and increased risks of suicide.

6. QSuper would like to partner with you to enable a safe and sustainable return to work, when medically suitable. QSuper can aid via Occupational Rehabilitation to help plan and facilitate suitable return to work goals. Occupational Rehabilitation can also provide strategies to help uplift work-readiness and provide additional support to Cory Shepherd through their return-to-work journey.

a. Is the engagement of an occupation rehabilitation provider likely to assist in facilitating a return to work in the next 1–2 months?

No, the engagement of an occupational rehabilitation provider is not likely to assist in facilitating a return to work in the next 1-2 months due to ongoing symptoms, impaired decision-making ability, and judgement.

b. If no, advise why and when you consider this assistance would be appropriate, or alternatively what strategies or support is being or could be utilised to enable a successful return to work?

The assistance of an occupational rehabilitation provider would not be appropriate at this time due to the following reasons:

1. Current mental health status: Cory is experiencing significant psychological distress due to multiple life stressors, including relationship breakdown, job loss, and bereavement.

This information was disclosed for the only reason of clinical information and not for medico-legal use. If a patient or any third party requests a copy, please direct them to our office. This message contains confidential information and is intended only for the individual named. If you are not the named addressee, you should not disseminate, distribute or copy this document. Please notify the sender immediately if you have received this document by mistake and delete that from your system.

2. Ongoing legal issues: Corey is involved in unfair dismissal proceedings with his employer, Queensland Health. This unresolved situation may complicate any return-to-work plans.

3. Need for stabilisation: The current focus is on stabilising Cory's mental health through medication management and psychological interventions

4. Recommendation to defer major decisions: I have advised Cory to defer in making decisions regarding work settlement or legal matters until his mental state improves.

Occupational rehabilitation assistance would be more appropriate once Cory's mental health has stabilised and the legal issues with his former employer have been resolved. This may take several months. Current strategies and support being utilised to enable a successful return to work in the future include:

1. Medication management: Restarting Vyvanse 30 mg daily to assist with focus and cognitive function, alongside continuing Fluoxetine for mood symptoms.

2. Psychological support: Ongoing counselling with Catherine, focusing on coping strategies and emotional regulation and focused CBT/trauma informed therapy

3. Lifestyle interventions: Implementation of a strict daily routine, including self-care activities and sleep hygiene.

4. Regular psychiatric review: A follow-up appointment is scheduled in 4 weeks to assess response to medication changes and routine implementation.

Once Corey's mental health has stabilised and legal issues are resolved, engagement with an occupational rehabilitation provider could be considered to facilitate a gradual return to work.

7. Do you have any additional comments to add not covered in the above?

Cory described himself as detail-oriented, perfectionistic, and having high standards. He tends to hyperfocus on tasks and can become consumed by issues he perceives as unjust. Cory's mother described him as clever, talented, and determined. She reported feeling undervalued and frustrated with perceived inconsistencies in policy implementation and described as having a strong work ethic. However, he struggles with assertiveness and decision-making, particularly in workplace and relationship contexts.

Please feel free to contact me on 0756545580 if you have any questions.

Yours sincerely,



Dr Ravikumar Bangalore Krishnaiah

MBBS, MRCPsych (UK), CCT (General Adult), CCT (Old age/Dementia), CCT (Addiction), FRANZCP, MBA (UK), AFRACMA, MPH (UNSW)

Provider number: 473280RT/473280QF

Consultant General Adult/Older person and Addiction Psychiatrist, Specialist in Adult Neurodevelopmental Disorder and Dementia

Expert in management of ADHD/ASD/Learning Difficulties/Acquired Brain injury and Dual diagnosis and complex case management

TAB M5

The question put to the treating psychiatrist, and the report he identified as his answer: Major Depressive Disorder with anxious distress, presenting as "workplace stress stemming from issues with management and rostering at Queensland Health" (5 to 8 September 2026)

Where the Respondent holds it

Not on the Respondent's amended List of Documents. The notice at the front of this tab is the Respondent's own notice of non-party disclosure to the practice, sealed 4 July 2025. Served with this bundle.

Pages included

Four documents, in this order. (1) The Respondent's Form 29 notice of non-party disclosure to the Mind and Memory Service, sealed 4 July 2025, pages 1 and 2, being the sealed cover and the page stating the three matters in issue. (2) The Appellant's email to Dr Krishnaiah of 5 September 2026 at 10:50 am, asking him to address the three matters in issue named in the Respondent's notice of non-party disclosure - whether the Appellant sustained a personal injury, whether it arose out of or in the course of employment, and whether employment was a significant contributing factor - from his own assessments and records (pages 5 to 7 of the chain as printed). (3) The end of Dr Krishnaiah's reply of 5 September 2026 at 12:04 pm: "I can provide all medical records I have to you. You can use them according to the need to support your legal issues" (page 4 of the chain). (4) Dr Krishnaiah's reply of 8 September 2026 at 7:18 am: that he had attached "the report that captures the relevant information you have requested" (page 1 of the chain). The report he attached is the report of 13 February 2025 at Tab M4, and is not reproduced here. In it, at page 1: the diagnosis, "Major Depressive Disorder with anxious distress (DSM 5- 296.23)"; that he "presented with workplace stress stemming from issues with management and rostering at Queensland Health"; that "These issues began approximately one year ago when a new manager was appointed"; that after raising concerns about rostering and fatigue management "he experienced changes in work conditions including shorter breaks, more night shifts"; and that "His pay was withheld or delayed for up to five months at a time, leading to significant financial stress". No report has been prepared for this proceeding.

Pages omitted, and why

Pages 2 and 3 of the email chain, being the Appellant's reply of 5 September 2026 at 1:16 pm and the first part of Dr Krishnaiah's message of 12:04 pm, which are not relied upon; and pages 3 to 6 of the Form 29, being the schedule of documents sought and the remaining form pages.

This tab is added so that the notice the question came from, the question, the author's permission and the answer can be read together with the report at Tab M4, which precedes it. Schedule row 4 states what the report is, and is not, relied upon for.



Form 29 - Notice of non-party disclosure

Industrial Relations Act 2016, section 989

Industrial Relations (Tribunals) Rules 2011, rules 64A, 64B, 64C, 64D, 64E, 64F, 64G, 64H and 64I

Information

- Use this form to obtain documents from non-parties.
- Please read this form carefully. Please complete all relevant sections. Information that is missing or non-compliant with the relevant section of an Act or the Rules may result in the rejection of your form.
- Once this form has been processed the Industrial Registry will contact you so that you can collect the authorised forms and serve them on the relevant person.

For further information please contact the Industrial Registry on 1300 592 987 or via email at qirc.registry@qirc.qld.gov.au

If there is an affected party and Rule 64D applies, the lawyer must complete certificate in Form 31 if the affected party is not to be served.

Applicant/Appellant:	Cory Shepherd

V

Respondent:	Workers' Compensation Regulator

PLEASE NOTE: If there are more than two parties to this application, please complete a **Form I - Parties list** and file it with this form.

ISSUED WITH THE AUTHORITY OF THE  INDUSTRIAL COURT OF QUEENSLAND  QUEENSLAND INDUSTRIAL RELATIONS COMMISSION

Filed in the Industrial Registry on (Date):	04 / 07 / 2025
Upon the request of	
Name of requesting party:	Workers' Compensation Regulator
Address:	150 Mary Street, Brisbane QLD 4000
Signature of Industrial Registrar:	 
Date:	04 / 07 / 25

To:	
Name of nominated party:	The Practice Manager, Mind and Memory Service
Address:	Shop 6, Upper floor, 21 Coomera Grand Drive, Upper Coomera QLD 4209
And to:	
Name of affected party:	
Address:	

NOTE: If there is more than 1 affected party - please complete schedule 1

Between 7 and 14 days after the day of service of this notice on you, you must (for the purpose of the Court or Commission determining this proceeding) produce the document(s) mentioned or described in the schedule to the requesting party, in accordance with this notice.

The matter in issue in the proceedings about which the document(s) sought is/are directly relevant is as follows:

3000 character limit. (Note: If more than 3000 characters are required please attach a schedule)

Matters in issue in the proceedings relate to s32(5) of the *Workers' Compensation and Rehabilitation Act 2003* (the Act). EG:

1. Did Mr Shepherd sustain a personal injury

2. Did the personal injury arise out of or in the course of Mr Shepherd's employment

3. Was Mr Shepherd's employment a significant contributing factor to the injury

The matters in issue in the proceedings about which the documents sought are directly relevant are 1 to 3 above.



"I acknowledge the Kombumerri people of the Yugambah language region as the Traditional Custodians of the land on which I live and work, and pay my respects to Elders past and present."

From: Cory Shepherd

<coryshepherd1@hotmail.com>

Date: Saturday, 5 September 2026 at 10:50 am

To: Ravikumar

<ravikumar@mindandmemoryservice.com.au>

Cc: Info M

<info@mindandmemoryservice.com.au>

Subject: Cory Shepherd (DOB 11.01.1991) — one-page report by 9 Sep 2026 if possible (pay / deposit)

Dear Dr Krishnaiah,

I am copying Mind and Memory Service so the practice has the same materials.

I need a short one-page report for my QIRC appeal. The Commission's deadline to serve any expert report is 4:00 pm, Wednesday 9 September 2026. I am self-represented, so without it I have nothing to serve.

I am happy to pay for the report and for your time, and I can put a deposit now if that helps. Please tell me the fee, how much deposit you would like, and how to pay. I may only be able to afford a deposit on the amount currently as my employer has not paid me for some time and I no longer receive any funds from my super.

WHAT HAS BEEN REQUESTED (THE THREE QUESTIONS)

These are the matters named on the Workers' Compensation Regulator's Notice of Non-Party Disclosure (attachment 1, sealed 4 July 2025):

1. Did Mr Shepherd sustain a personal injury?
2. Did the personal injury arise out of or in the course of Mr Shepherd's employment?
3. Was Mr Shepherd's employment a significant contributing factor to the injury?

A short report answering those three is all that is required.

HOW I ASK FOR THEM TO BE ANSWERED

Please answer from your own assessments, consultations, and records. It does not need to be a formal medico-legal report, and no further review is needed. Please ignore capacity questions 3.4–3.8 in my 17 August letter (still inside attachment 2) — not needed for this deadline. Nothing here asks you to comment on anyone else's opinion.

In practice, those three matters break down to these five points:

1. Diagnosis, and date of onset as best the clinical record shows. My appeal pleads onset on or about 18 June 2024. The work events I rely on start with the shifts of 17–18 March 2024. If your record puts onset elsewhere, please say so.
2. Any pre-existing condition or vulnerability — and if so, whether the enclosed material aggravated it, and to what extent.
3. Whether the injury arose out of, or in the course of, employment — including fatigue carry-over from the roster (the seven-hour break between those March shifts against the ten hours required; MET calls that period; no fatigue risk assessment for the role before 30 June 2024), plus further rostering problems and unresolved follow-up. The dated list is in the

chronology (attachment 3) and on page 1 of attachment 2.

4. Non-employment factors in your 13 February 2025 report (relationship breakdown, job loss, bereavement) — and when each sits relative to onset.
5. Whether employment was a significant contributing factor to the injury.

Please also note briefly what you relied on.

ATTACHMENTS — READ IN THIS ORDER

1. Attachment 1 — the Notice (the three questions above).
2. Attachment 3 — one-page chronology.
3. Attachment 2, page 1 finding aid if you need a page reference (bookmarks / JUMP TO for the rest).

You already hold your clinical file and the report of 13 February 2025.

PLEASE CONFIRM

1. Can you do this by 4:00 pm Wednesday 9 September 2026?
2. If not, what firm date can you give?
3. Are you still waiting on Queensland Health — if so, what was requested, and when?
4. Fee, deposit amount, and how to pay.

If Wednesday is not possible, I can ask the Commission for more time, but I need a firm date first.

Thank you,
Cory Shepherd

0417 400 227

coryshepherd1@hotmail.com

Sorry for that

I can provide all medical records I have to you. You can use them according to the need to support your legal issues.

QH- I have not received response yet

Kind regards

Ravi

Dr Ravikumar Bangalore Krishnaiah

MBBS, MRCPsych (UK), CCT (General Adult), CCT (Old age/Dementia), CCT (Addiction), FRANZCP, MBA (UK), AFRACMA, MPH (UNSW)

Provider number: 473280UL/473280RT

Consultant General Adult/Older person and Addiction

Psychiatrist, Special interest in Adult Neurodevelopmental Disorder and Dementia

Mind and Memory Service

19 Harbour Village Parade, First Floor, Building 5 D, Coomera

QLD 4209, Australia

Website <http://www.mindandmemoryservice.com.au>

Landline [07 5654 5580](tel:0756545580)

Fax [07 5636 1005](tel:0756361005)

Email: info@mindandmemoryservice.com.au

For all medical emergencies or crisis please call below helplines as our clinic does not provide emergency care

Emergency 000, Lifeline [13 11 14](tel:131114), Beyond Blue [1300 224 636](tel:1300224636)

Suicide Call Back Service [1300 659 467](tel:1300659467), Mens Line Australia [1300 789 978](tel:1300789978)

Butterfly Foundation [1800 334 673](tel:1800334673), Health Direct [1800 022 222](tel:1800022222)

Child and Youth Mental Health [1800 048 636](tel:1800048636), Headspace [1800 650 890](tel:1800650890)

Kids Helpline [1800 551 800](tel:1800551800), Kids Helpline Parents [1800 654 432](tel:1800654432)

Alcohol and Drug Support [1800 198 024](tel:1800198024), Blue Knot Foundation [1300 657 380](tel:1300657380)

Friendline [1800 424 287](tel:1800424287), Head to Health [1800 595 212](tel:1800595212)

Mind Spot [1800 614 434](tel:1800614434), SANE Australia [1800 187 263](tel:1800187263)

1800RESPECT [1800 737 732](tel:1800737732), Grief Line [1300 845 745](tel:1300845745)



07 5654 5580

Fax - 07 5636 1005
info@mindandmemoryservice.com.au
mindandmemoryservice.com.au
[@mindandmemoryservice](https://www.instagram.com/mindandmemoryservice)



**Re: Cory Shepherd (DOB 11.01.1991) —
thank you, and yes please to the records**

From: Ravikumar

ravikumar@mindandmemoryservice.com.au

To: Cory Shepherd coryshepherd1@hotmail.com

Cc: Info M info@mindandmemoryservice.com.au

Sent: Tuesday 8 September at 07:18

Hi Cory,

Due to staff sickness, I am not able to get all the documents to you.

I have attached the report that captures the relevant information you have requested.

Let me know if this is okay.

Kind regards

Ravi

Dr Ravikumar Bangalore Krishnaiah

MBBS, MRCPsych (UK), CCT (General Adult), CCT (Old age/Dementia), CCT (Addiction), FRANZCP, MBA (UK), AFRACMA, MPH (UNSW)

Provider number: 473280UL/473280RT

Consultant General Adult/Older person and Addiction

Psychiatrist, Special interest in Adult Neurodevelopmental

Disorder and Dementia

Mind and Memory Service

19 Harbour Village Parade, First Floor, Building 5 D, Coomera

QLD 4209, Australia

Website <http://www.mindandmemoryservice.com.au>

Landline [07 5654 5580](tel:0756545580)

Fax [07 5636 1005](tel:0756361005)

Email: info@mindandmemoryservice.com.au

**For all medical emergencies or crisis please call below helplines
as our clinic does not provide emergency care**

Emergency 000, Lifeline [13 11 14](tel:131114), Beyond Blue [1300 224 636](tel:1300224636)

Suicide Call Back Service [1300 659 467](tel:1300659467), Mens Line Australia [1300 789 978](tel:1300789978)

Butterfly Foundation [1800 334 673](tel:1800334673), Health Direct [1800 022 222](tel:1800022222)

TAB M6

Clinical records of Dr Krishnaiah from 24 October 2024

Where the Respondent holds it

Offered by the practice on 5 September 2026 and requested. Not yet received.

Pages included

None at the date of service. The records will be served on receipt, and any direction required for reliance on them will be sought.

Pages omitted, and why

Not applicable.

Schedule row 5 states what this document is, and is not, relied upon for.

TAB M7

Employee Capability Checklist completed by Dr Day Hong Ma, 3 July 2026

Where the Respondent holds it

Not on the Respondent's list. Served with this bundle.

Pages included

All five pages.

Pages omitted, and why

None.

Schedule row 6 states what this document is, and is not, relied upon for.

Employee Capability Checklist – Cory Shepherd

*** Please note that a separate medical certificate is also required to support the completed Capability Checklist.*

Information provided in the Capability Checklist below will assist our Organisation to determine appropriate tasks in the event that the identified worker is cleared for suitable duties. Please ensure that all sections of the Capability Checklist are completed in full.

I have examined Cory Lea Shepherd (DOB 11 January 1991) and consider that they are diagnosed with the following condition:

Major Depressive Disorder with anxious distress (DSM-5 296.23) — work-related psychological injury, onset 18 June 2024.

- ☒ I have viewed the worker's Position Description (attached — Administration Officer, Switchboard Services (AO3), Logan Hospital) and/or
- ☒ I am aware of the requirements of their role

It is my opinion that they are fit to resume:

- ☐ Normal duties
- ☒ Modified duties
- ☒ Reduced hours (hours/days)
- ☐ Workplace Performance Discussions

Modified duties relate to:

- ☒ Tasks performed
- ☒ Duration

Hours to be worked

Week 1: 8 hours/day 3 days/week	Week 5: 8 hours/day 3 days/week
Week 2: 8 hours/day 3 days/week	Week 6: 8 hours/day 3 days/week
Week 3: 8 hours/day 3 days/week	Week 7: 8 hours/day 3 days/week
Week 4: 8 hours/day 3 days/week	Week 8: 8 hours/day 3 days/week
(6 shifts per 10-shift fortnight: max 2 weekday day/afternoon + 2 night + 2 weekend shifts — continuation of the reduced pattern Mr Shepherd has sustained since 2025)	

Fit to commence the above from: 3 July 2026 (continuation of existing arrangement)

Date of next review: 28 August 2026 (approximately 8 weeks)

Activity	No capacity	Restricted / As Tolerated	Normal	Details of any restrictions and reasons
COGNITIVE				
Learning new tasks	☐	☐	☒	
Ability to initiate tasks independently	☐	☐	☒	
Understanding instructions	☐	☐	☒	
Carrying out instructions accurately	☐	☐	☒	
Maintain focus/concentration	☐	☒	☐	Concentration reduced during sustained high-stress periods; managed by rostering to lower call-volume shifts.
Executing tasks within a reasonable timeframe	☐	☐	☒	
Working autonomously without supervision	☐	☐	☒	
Prioritising tasks	☐	☐	☒	
Making decisions	☐	☐	☒	
Multi-tasking	☐	☒	☐	Concurrent high-volume demand exacerbates symptoms; suitable on lower-volume shifts.
Memorising information	☐	☒	☐	Working memory affected under stress (treating psychiatrist report 13-Feb-2025).
Working in a high demand environment	☐	☒	☐	Limit weekday day/afternoon shifts — which carry the peak call volume and the majority of complaints — to no more than 2 per fortnight; remaining shifts rostered to the lower-volume weekend (2) and night (2) shifts.
RELATIONAL				
Communicating with internal/external customers	☐	☐	☒	
Working in a team environment	☐	☐	☒	
Handling conflict	☐	☒	☐	No complaint-handling duties: complaints received at the Switchboard are to be logged and redirected to management/the dedicated complaints area, not actioned, resolved or absorbed by Mr Shepherd.
Supervising others	☐	☐	☒	Not a requirement of the AO3 role.
Interacting appropriately with others	☐	☐	☒	
Do you foresee any existing stressors that could impact on a return to substantive role? If so, please provide additional comments:				Yes. (1) Complaint-handling exposure at the Switchboard; (2) being held accountable, and blamed, for failures that are the responsibility of others across the hospital, regardless of where the fault lies; (3) rostering and fatigue-management practices; (4) unresolved workplace matters involving line management. Recommend return-to-work communications and this checklist be managed through the Injury Management Team Human Resources.

Medications

Is the worker currently taking any medication?

☒ Yes

☐ No

Please outline any side effects from medication currently being taken that may impact on a person's functional capacity or task performance (for example drowsiness, dizziness, confusion, impaired concentration, fatigue, slurred speech):

Current medications:

1. Fluoxetine 60 mg mane (3 × 20 mg capsules).
2. Lisdexamfetamine (Vyvanse) 30 mg mane — for cognitive symptoms and focus.

Quetiapine (Seroquel) 25 mg nocte, previously prescribed for rumination and sleep, has been ceased.

Relevant effects on task performance: maintaining a regular sleep routine remains central to symptom management following cessation of the night-time medication. This supports (a) limiting night shifts to no more than 2 per fortnight, (b) predictable rostering published in advance, and (c) a minimum 10-hour break between rostered shifts consistent with the applicable Award and fatigue risk management standards. Residual symptoms include reduced concentration and working memory under sustained high-demand load and fatigue sensitivity.

Additional Comments:

1. Please elaborate on the current medical condition Mr Shepherd is suffering from?

Major Depressive Disorder with anxious distress (DSM-5 296.23), a work-related psychological injury with onset 18 June 2024. Treatment is ongoing (fluoxetine, lisdexamfetamine) under psychiatric care. Residual symptoms most relevant to work capacity are reduced concentration and working memory under sustained high-demand load, fatigue sensitivity, and symptom exacerbation on exposure to the identified workplace stressors (complaint handling; being held accountable and blamed for the failures of others; unpredictable rostering).

2. In your opinion do you feel that Mr Shepherd's current condition is temporary, episodic or permanent in nature? What is the expected duration of the current restrictions?

Episodic, with symptom exacerbation linked to stressor exposure. The restrictions below are expected to remain necessary for at least the next 3–6 months, subject to treatment response and 8-weekly review.

3. Based on your current assessment, what work capacity (including number of shifts, hours, and type of duties) would best support Mr Shepherd's recovery while minimising risk of exacerbation?

Six 8-hour shifts per fortnight (of the ten rostered in a full-time fortnight), comprising:

(a) No more than two weekday day/afternoon shifts. These shifts carry the peak call volume and the majority of complaints — the identified stressors. Retaining two per fortnight maintains graded exposure to the full role and preserves work capacity; the cap prevents multiple high-load shifts accumulating within a fortnight, which is the pattern associated with symptom exacerbation.

(b) Two weekend day shifts. Weekend call volume and complaint exposure are materially lower while the duties are identical, allowing the same work to be performed at an intensity consistent with Mr Shepherd's current concentration and working-memory capacity, and preserving a normal sleep-wake cycle.

(c) No more than two night shifts at this stage. Night shifts are low in volume and complaints, but a stable sleep routine is central to symptom management following cessation of the night-time medication; capping nights protects that routine and limits cumulative fatigue. This cap is suitable for reassessment at review.

This reduced pattern of approximately six shifts per fortnight is the pattern Mr Shepherd has in fact worked and tolerated over the past twelve months without deterioration, and maintaining rather than increasing it best supports recovery while minimising risk of exacerbation. Usual switchboard operational duties remain suitable; complaint-handling duties are excluded as below.

4. Are there any specific workplace modifications or supports you would recommend (e.g. reduced workload, predictable rostering, supervision level, reduced exposure to stressors) to assist Mr Shepherd during this period?

(a) No complaint-handling duties — complaints received at the Switchboard to be logged and redirected, not actioned or resolved by Mr Shepherd; (b) predictable rostering published in advance, with a minimum 10-hour break between shifts consistent with the Award and fatigue risk management standards; (c) return-to-work coordination and correspondence through the Injury Management team, Human Resources; (d) access to EAP (TELUS Health) as an available support; (e) review in approximately 8 weeks.

Please complete this form and return to the Injury Management team, Logan & Beaudesert Health Service, Human Resources by email lbh_injurymanagement@health.qld.gov.au

Please include your practice details/stamp here

Signature

Date



3-7-26.

**MY DOCTORS CLINIC
OPEN 24 HOURS**

Dr Day Hong Ma

MBBS,FRACGP,FACRRM Provider No: 4696842K

16/3221 Surfers Paradise Blvd

Surfers Paradise, QLD 4217

Ph: 07-5592 2299 Fax: 07-5539 9188

www.mydoctorsclinicsurfers.com.au

TAB M8

Review Decision 69983, reasons for decision dated 24 October 2024, pages 17, 26 and 27 of 28

Where the Respondent holds it

Respondent's amended List of Documents item 4. Contents admitted by the Respondent on 18 February 2026.

Pages included

Page 17 (the medical evidence and the finding that employment was a significant contributing factor), and pages 26 and 27 (the findings on management action and the conclusion).

Pages omitted, and why

The remaining 25 pages, which the Respondent holds and which are not relied upon for the purposes stated in the schedule.

Schedule row 7 states what this document is, and is not, relied upon for.

Reasons for decision

The work capacity certificate of Dr Hawes dated 1 July 2024 indicated your mechanism of injury was consistent with his clinical findings and there was no pre-existing factor or condition. This was maintained in all later work capacity certificates.

According to the communications report, in a conversation with WorkCover on 2 September 2024, Dr Hawes stated that work events were the sole cause of your psychological injury.

Having regard to the medical evidence, I am satisfied your employment was a significant contributing factor to the psychological injury.

It is now necessary for me to determine whether your injury has arisen out of or in the course of management action, and if so, whether management action is found to be reasonable and taken in a reasonable way.

Did the injury arise out of or in the course of management action?

The term 'management action' is not defined in the Act. It has been interpreted by the Courts broadly and will vary depending on the circumstances of the case. The action must relate to action taken by or on behalf of the employer in connection with the worker's employment.

In *Avis v WorkCover Queensland*², the Industrial Court held that the phrase 'arising out of' involves a causal or consequential relationship between the management action and the injury, but does not require a direct or proximate relationship.

Factor 2: Management action taken in response to your complaint

Submitting complaints regarding aspects of your employment is connected to your employment. The complaint was reviewed by the Ethical Standards Unit, and then by Human Resources. These departments have the power to take action in response to your complaint on behalf of the employer. I find the steps taken by them were taken on behalf of the employer in connection with your employment and therefore amounted to management action.

Factor 3: Withholding of pay

The pay issues arose as a result of Ms Taylor refusing your claims for payment in specific situations. Ms Taylor was your direct manager and thus in a superior position, which provided her with the power to manage your employment. Receiving payment for your work is directly

² [2000] QIC 67

Reasons for decision

In your response dated 9 August 2024, you stated you were expected to finish on 17 March 2024 at 11:00 pm and start again on 18 March 2024 at 6:00 am. This meant you received only a 7-hour break between the shifts, which did not include travel time. If you factored in travel time, your break between shifts would have been less than 5 hours. You reiterated it was unreasonable for Ms Taylor to apply the 8-hour agreement you signed earlier in this situation.

The response of the employer dated 6 September 2024 submitted that during pay period 20, commencing on 4 March 2024, you were scheduled for 3 night shifts with appropriate rest days allocated. You arrived 15 minutes late on 17 March 2024. The employer wrote that records indicate Ms Taylor took appropriate action by shortening your shift, but this resulted in you being remunerated for the 30 minutes you did not work. The employer did not mention your shift on 18 March 2024. They also confirmed you had not formally rescinded your agreement for only an 8-hour break, which you signed on 17 June 2020, and the change of your contracts and adjustments in working hours did not invalidate this agreement. This meant the agreement remained current. In their response, the employer included an extract of the Hospital and Health Services General Employees (Queensland Health) Award, which stated that employees must be provided with a break of not less than 10 hours between the termination of one shift and the commencement of another shift, and 8 hours applied instead of 10 only in specific circumstances.

In considering the evidence, I find there was uncertainty between you and the employer regarding whether the 8-hour agreement continued to apply. However, it is not necessary for me to consider whether the employer's stance on its application was reasonable. The break between the shift on 17 March 2024 and 18 March 2024 equated to 7 hours. Even if you were allowed to leave early on 17 March 2024 as suggested by the employer, you left a maximum of 30 minutes early, which meant you still did not receive a minimum 8-hour break. Based on this, I find the rostering of these two shifts amounted to unreasonable management action given that it was in direct contradiction to the award and the 8-hour agreement.

Conclusion

After considering the evidence available at review, in relation to the factors for determination, I have determined:

- you sustained a personal injury of a psychological nature
- your injury arose out of employment, to the extent that it arose out of factors 2, 3 and 4, where employment was a significant contributing factor
- factors 2 and 3 amounted to reasonable management action taken in a reasonable way

Reasons for decision

- factor 4 amounted to unreasonable management action.

In the decision of *Delaney v Q-COMP*⁵ the Industrial Court held that in determining the issue of whether management action was reasonable and taken in a reasonable way, a global evaluation of management actions must be made. In *Delaney*, it was held that when management actions are joined by subject matter, time and personality, the actions are entitled to be evaluated on a global basis.

I note that since two out of the three causative factors amounted to reasonable management action taken in a reasonable way, this means your psychological injury mainly arose out of such management action. Further, factor 4 was initially only substantiated to the extent of one incident and it was only the one incident which amounted to unreasonable management action. Therefore, I find overall your psychological injury arose out of reasonable management action taken in a reasonable way and section 32(5) is consequently enlivened.

Summary

I therefore conclude that the provisions of section 32(5) of the Act exclude your psychological condition from the definition of 'injury' within section 32(1) of the Act.

Therefore, I have determined that you did not sustain an 'injury' in accordance with section 32 of the Act and you do not have an entitlement to compensation.

Appeal rights

If either party disagrees with this decision then either party may appeal to the Queensland Industrial Relations Commission in Brisbane. Either party has 20 business days from the date of receipt of this decision in which to lodge an appeal. A copy of the notice lodged with the Commission must also be served on the Workers' Compensation Regulator within 10 business days.

For information about the appeal process please contact me or refer to:

<https://www.worksafe.qld.gov.au/claims-and-insurance/compensation-claims/appealing-a-review-decision/guide-to-appealing-a-review-decision>.

⁵ [2005] QIC 11; 178 QGIG 197

TAB M9

The Respondent's responses of 18 February 2026 to the Appellant's first notice to admit, so far as they concern the medical documents

Where the Respondent holds it

The Respondent's own document, served by it on 18 February 2026.

Pages included

Page 4 of the notice (items 32 to 38: the 16 November 2023 entry, the Hawes certificate, the Review Decision finding and the Krishnaiah diagnosis) and page 9 of the response, which carries the Respondent's answers to those items and in which the Respondent admits the contents of the Review Decision, and for the other documents admits that each exists and says what it says while reserving accuracy and relevance. Two answers on that page go further and are set out as they stand: at row 32 the Respondent denies the fact put about Exhibit A5 on the ground that the records show a past medical history of anxiety and ADHD from 26 October 2022, and at row 33 it admits the content of the certificate but puts its date as 8 September 2024.

Pages omitted, and why

The remaining pages of the notice and response, which concern other subjects.

This tab is added so that the Respondent's own position on the documents at Tabs M1, M2, M4 and M8 sits with them.

No.	Fact to be Admitted (with Exhibit Reference)	Admit / Deny
32	On 14 July 2025 , the Respondent's representative (Renee Matheson) emailed the Appellant stating she had " <i>received last week</i> " documents but did not disclose possession of Exhibit A5 (Exhibit H1) .	
33	Exhibit A5 (p. 52) contains a consultation record dated 16 November 2023 authored by Dr. Priyal De Silva Nanayakkara.	
34	The entry dated 16 November 2023 explicitly states: " <i>No psychological illness such as depression/ psychosis.</i> "	
35	The Respondent is not in possession of any contemporaneous medical evidence prior to 2024 that contradicts the finding in Item 34.	
36	The Work Capacity Certificate issued by Dr. Peter Hawes dated 1 July 2024 (Exhibit A1) identifies the mechanism of injury as " <i>ongoing breaking of workplace rules by bosses, victimizing him</i> ".	
37	In the Review Decision (Exhibit B1 , p. 16), the Regulator stated: " <i>I am satisfied your employment was a significant contributing factor to the psychological injury</i> ".	
38	Dr. Ravikumar Krishnaiah (Psychiatrist) diagnosed the Appellant with Major Depressive Disorder on 13 February 2025 (Exhibit A4) .	

	Respondent's Response
26.	With respect to paragraphs 26 to 29 denies the fact that details contained in the records are not relevant because, read as a whole, the medical records are relevant in that they tend to prove or disprove an allegation in the proceeding.
27.	With respect to paragraph 30, admits the fact that the Respondent served the medical records on the Appellant's solicitors on 22 July 2025 and says further the Respondent was not notified that the Appellant's solicitors no longer acted for the Appellant until 29 July 2025.
28.	With respect to paragraph 31, does not admit the facts because these are matters within the Appellant's knowledge.
29.	With respect to paragraph 32, admits that the Respondent sent an email directly to the Appellant on 14 July 2025 and says that the email addressed to the Appellant on that date was in error and that the email should have been sent to the Appellant's solicitors, because he was legally represented. The Respondent states that the medical records were not in the possession of the Regulator at the time of sending the email on 14 July 2025.
30.	With respect to paragraph 33, admits the fact that there is an entry from Dr Nanayakkara in the records but does not admit the accuracy of the record.
31.	With respect to paragraph 34, admits the entry is listed in the record, but does not admit the accuracy of the entry.
32.	With respect to paragraph 35, denies untrue the fact because the records show a past medical history of Anxiety and ADHD from 26 October 2022 which is missing from Exhibit A5.
33.	With respect to paragraph 36, admits the fact Dr Peter Hawes provided a medical certificate stating that the mechanism was "ongoing breaking of workplace rules by bosses, victimizing him" but denies that as a fact and further denies the date of the certificate is 1 July 2024 because the certificate is signed 8 September 2024.
34.	With respect to paragraph 37, admits the contents of the Respondent's review unit decision but says that this fact is not relevant because the appeal is a hearing de novo and it is for the Commission to determine. action
35.	With respect to paragraph 38, admits that the report does say that the Appellant was suffering from Major Depressive Disorder, but does not admit the accuracy of the report.
36.	With respect to paragraph 39, admits that the report does say: <i>"Given the complex workplace issues, ongoing WorkCover claim and disputed unfair dismissal proceedings and impaired decision making abilities it is unclear at this stage the work goal and return to work. I will further update on this after next consultation. Please note that premature exposure to the workplace is more likely result in significant deterioration in his mental health..."</i> , but does not admit the accuracy of the report.
37.	With respect to paragraph 40, admits the facts contained therein.
38.	With respect to paragraph 41, admits the fact that Ms Taylor submitted the AVAC on 28 May 2024 but also says that on 21 May 2024 Ms Taylor emailed the appellant stating she was waiting for payroll confirmation on a few issues and would submit an AVAC for the next pay run as soon as she received it.
39.	With respect to paragraphs 42 to 45, denies untrue the fact, because the comparator used does not represent a true comparator in the circumstances and because the roster was equitable.
40.	With respect to paragraph 46, admits the facts contained therein.

8. Request: the documents not admitted on 8 September 2026

SERVED on the Respondent. Email 2, attachment 3. Reply sought by Friday 18 September 2026.

CORY LEA SHEPHERD

15 Edmond Street, Coomera QLD 4209 | coryshepherd1@hotmail.com

Ms Renee Matheson

Senior Appeals Officer, Workers' Compensation Regulator

150 Mary Street, Brisbane QLD 4000

By email: Renee.Matheson@oir.qld.gov.au

Dated 9 September 2026

WC/2024/227 – Cory Lea Shepherd v Workers' Compensation Regulator

Request: the documents not admitted on 8 September 2026

Dear Ms Matheson,

I refer to the Respondent's responses of 8 September 2026 to the Form 24 and Form 25 notices and to my letter of today's date on the letter that accompanied them. Five facts are not admitted and the authenticity of fourteen tabs of Annexure A is disputed. This request arises only from that response: had the authenticity of the copies been admitted, as their contents were under the Form 24, none of these documents would need to be sought. The documents concerned are set out in the schedule to this letter. The contents of each, other than Tab 31, were admitted in the Form 24 response at the facts noted; what is disputed is the authenticity of the copy served from my records.

At the mention on 7 August 2026 the Commissioner indicated that, for a document held by Metro South Health, the starting point is to ask the Regulator to make inquiries and produce it, and that non-party disclosure follows only if the Regulator does not. This letter is that step, put as a choice so that the matter can be closed without an application, and before the Respondent's material is due on 30 September 2026.

For each document in the schedule, could the Regulator please, by Friday 18 September 2026, do one of the following:

- (a) produce the Regulator's or Metro South Health's copy of the document, or confirm the authenticity of the copy served by reference to that copy; or
- (b) admit, under rule 49, the authenticity of the copy served at that tab of Annexure A, so that it may be tendered without further proof of what it is.

If neither is done for any document by that date, I will seek that document by notice of non-party disclosure to Metro South Health limited to the documents named in the schedule, and I will refer to this correspondence if the question of the costs of that step arises.

The documents are listed in the schedule on the following page. No request is made in respect of fact 154 or Tab 24. For Tab 31, the request is for the spreadsheet of recorded MET calls for 17 to 18 March 2024 referred to in the letter of 5 June 2026 and for the March 2024 sheet of the workbook from which it is drawn, in native format and as printed.

Yours faithfully,



Cory Lea Shepherd

Appellant, self-represented

WC/2024/227 – Schedule to the letter of 9 September 2026: the documents not admitted on 8 September 2026

Tab	Document	Contents admitted at	Copy known to be held
31	Screen capture, "2024 Emergency Code Register.xlsx", March 2024 sheet, entries 16 to 20 March 2024	Facts 228 to 231 not admitted. Fact 268 admits Metro South Health's statement that "a spreadsheet of recorded MET calls is available for the period 17-18 March 2024"	Metro South Health (letter of 5 June 2026, K-LM26/729, Items 1 and 2)
1	Role description, AO3 Switchboard Services, Logan Hospital	Facts 1 to 13	Metro South Health (Attachment 2 to the Request for Medical Information of 31 July 2026)
5	Email, Ms Taylor to the Appellant, 27 September 2023, 1:52 pm, "Approved - Permanent Full Time FTE"	Fact 38	Metro South Health
6	Email, Ms Taylor to Logan Switch, 15 April 2024, 12:39 pm, "Afterhours Oncall Process - Switchboard"	Facts 49, 163	Respondent's amended List of Documents, attachment to item 25
17 to 19	Movement forms, 27 February, 17 April and 9 June 2026, approved by Mr Hughes	Facts 14 and 15	Metro South Health
20	Letter, Metro South Health to Commissioner Dwyer, 5 June 2026, K-LM26/729	Facts 263 to 268	The Commission's file
21	Email, Ms Forrest to the Appellant, 7 July 2026	Fact 224	Metro South Health
22	Consultation Paper - Proposed Rosters for Switchboard Services, November 2024	Facts 169 to 175	Metro South Health
23	Consultation outcome - Proposed Rosters for Switchboard Services, December 2024	Facts 167, 176	Metro South Health
30	Email, Ms Taylor to Logan Switch, 23 August 2023, 2:18 pm, "What's Chloe's Hours?!"	Facts 71, 72; quoted in Review Decision 69983 (fact 70)	Metro South Health; the Respondent's claim file
30A	Email, Ms Taylor to Logan Switch, 18 June 2024, 8:58 am, "Good morning Team."	Facts 85 to 87	Metro South Health